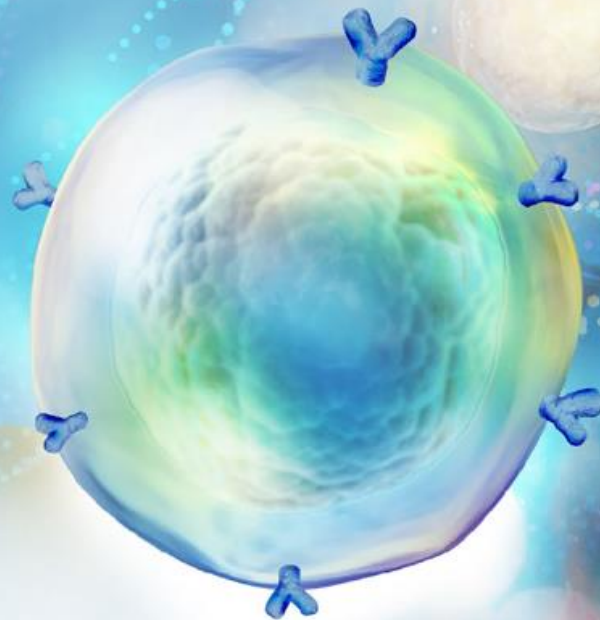




Management Presentation

IASO Bio

Jan 2026



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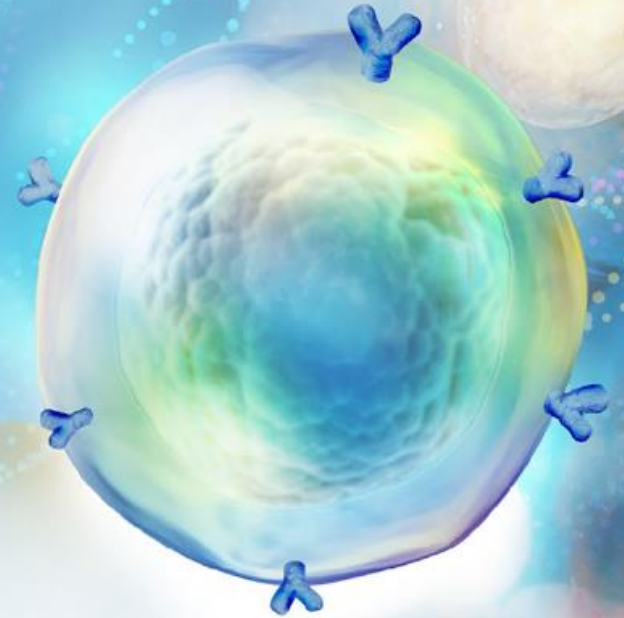
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Table of Content

SECTION 1	Company Overview	4
SECTION 2	Global Commercialization	14
SECTION 3	Pipeline Overview	18
SECTION 4	Manufacturing Capabilities	30
SECTION 5	Financial Overview	32
SECTION 6	Conclusion	35
APPENDIX A	Financial Statements and Use of Proceeds	38
APPENDIX B	Shareholding Structure	48
APPENDIX C	Market Data	50
APPENDIX D	Clinical Data	55
APPENDIX E	Corporate Strategy and Potential Milestones	57

SECTION 1

Company Overview



Our Aspiration and Vision

Focusing on innovative biotherapies to bring hope of cure to patients

We aspire to bring new and curative treatment in the fields of hematologic malignancies and autoimmune diseases

IASO Bio - An Innovative Pioneer in Advanced ATMPs



600+
Patients

Proven Outperformance of Core Product Eque-cel (CT103A)

The world's **first** fully human CAR-T therapy approved for marketing and the **first** BCMA-targeted CAR-T cell therapy approved in China

Safety and efficacy widely validated across over **600** patients in clinical trials and real world studies

With an ORR of **98.9%** and a CR/sCR rate of **88.4%**, along with a high MRD-negative rate of **98.9%**⁽¹⁾

10

Assets; Core Product
Included in the
National Commercial
Insurance Innovative
Drug Catalogue

Diversified, Comprehensive and Balanced Product Pipeline

1 commercialized core product, complementary by next-generation *in vivo* CAR-T assets in target patient populations

3 BLAs for the core product approved in Greater China ⁽²⁾, **3** applications accepted or submitted in Asia and the Middle East

9 pipeline candidates: **3** ex vivo CAR-T therapies, **5** *in vivo* CAR-T cell therapies, and **1** antibody candidate

5
Diversified
Platforms



Fully Human Antibody
Discovery Platform



High-throughput CAR-T
Cell Therapy Screening
and Optimization
Platform



In Vivo CAR-T
Platform



Clinical Development
and
Clinical Translational
Research Platform



Advanced
Manufacturing
and CMC Platform

Distinct Technology Platforms

70%
Market
Share

One of the most comprehensive commercialization platforms for ATMPs in China

Nationwide presence across over **100** leading hospitals, covering over **95** DTPs; nearly **70%** market share in China CAR-T cell therapy market for treatment of MM in 2024

Facilitated **overseas patient access (treatment in China or cross-border)** through patient referrals, medical tourism and Named-Patient Programs (NPPs)

Established collaborations with renowned biopharmaceutical companies:

Innovent

Sana

Cabaletta Bio

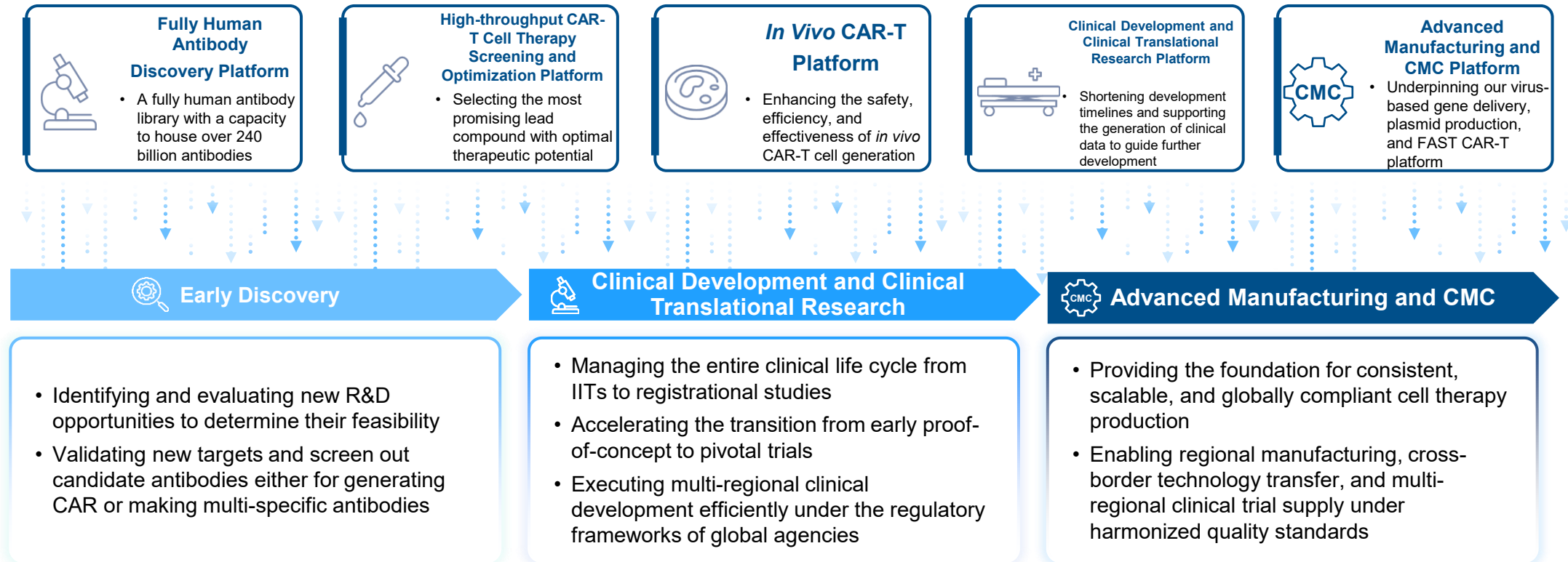
Umoja

Notes:

1. Source: Phase Ib/II pivotal trial (in patients naive to CAR-T treatment). ORR: objective response rate; CR: complete response; sCR: stringent complete response; MRD: minimal residual disease.

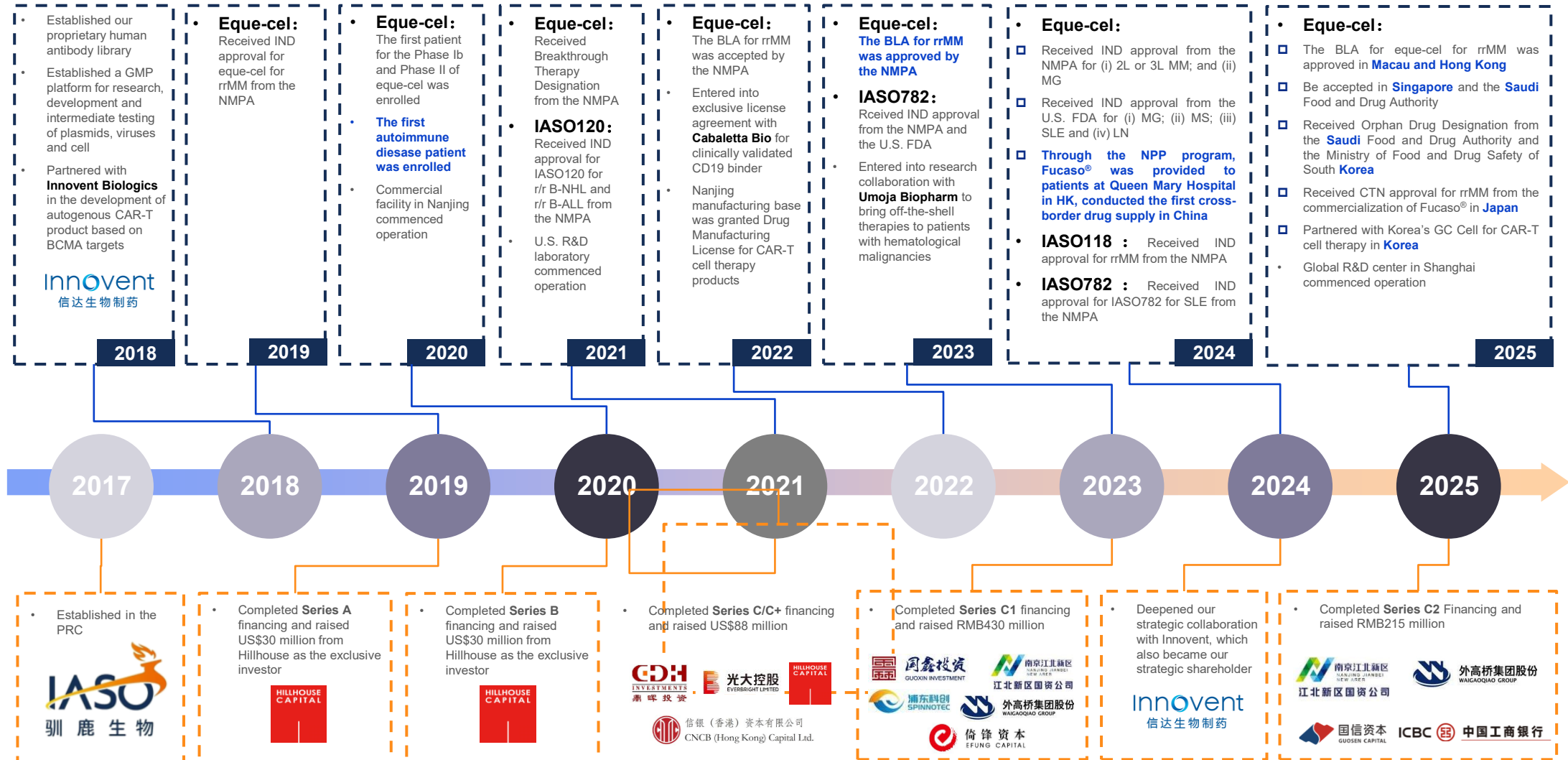
2. According to the Company's website, the BLA for its core product was approved by the Hong Kong Department of Health (DOH) on November 27, 2025, for the treatment of relapsed or refractory multiple myeloma (r/r MM).

Integrated End-to-end Development Capabilities Covering a Full Cycle of Innovative Drug R&D, and CMC and Clinical Development



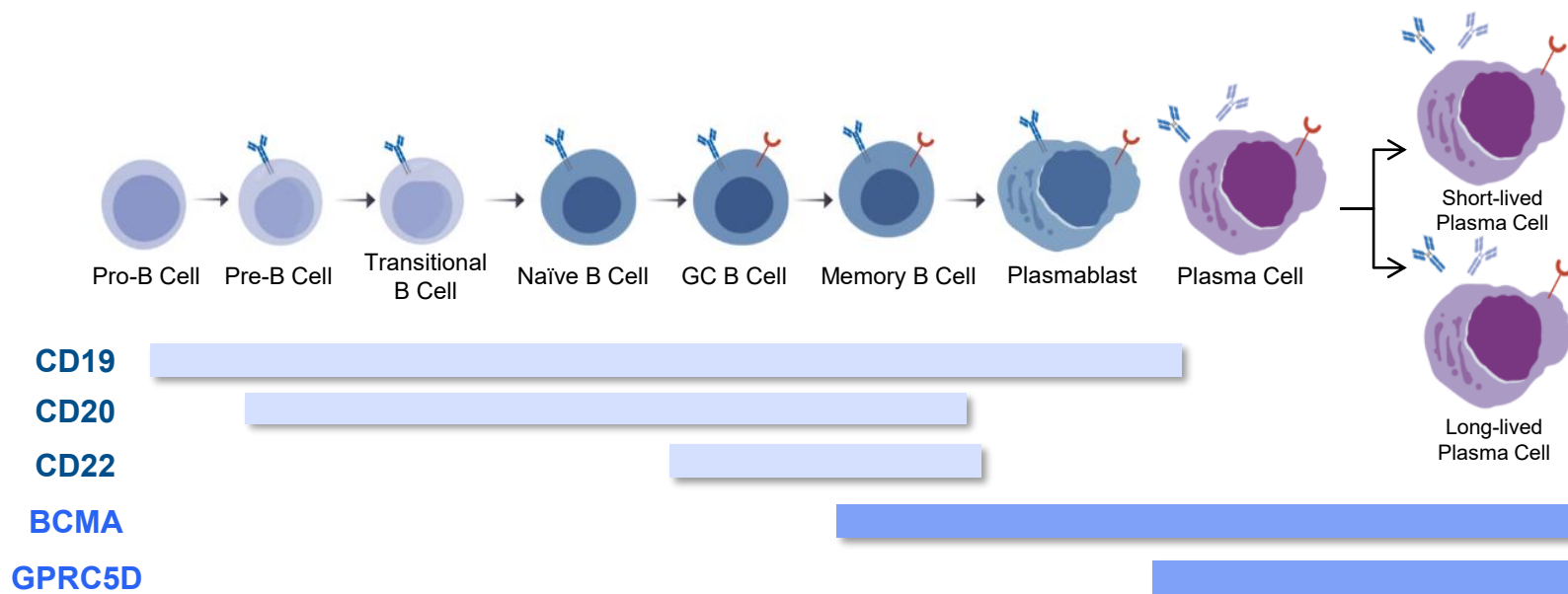
These capabilities position us among the few biopharmaceutical companies with fully integrated, end-to-end manufacturing competence starting from plasmid and lentiviral vector to final CAR-T product, laying the foundation for global scale-up and commercialization

Company Milestones



Focusing on B Cells Targeted Therapies and Their Applications

B Cell Development and Maturation & Key Targets



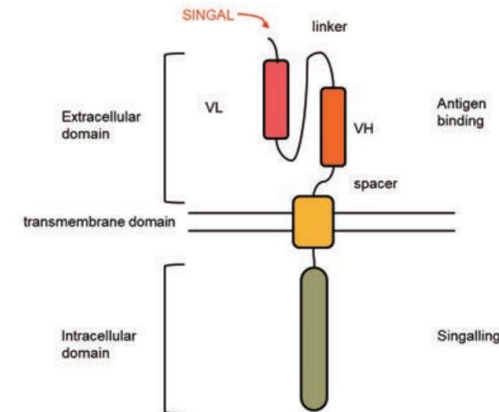
Hematological Malignancies

B cell related targets have emerged as a key therapeutic approach for hematologic malignancies, demonstrating significant efficacy in treating diseases such as MM, ALL and NHL

Autoimmune Diseases

B cells contribute to autoimmunity through autoantibody production, antigen presentation, and cytokine secretion, and their developmental stages provide distinct therapeutic opportunities

CAR-T Cell Therapy



- ✓ Comprise viable cells capable of in vivo expansion and persistence
- ✓ Enable deep and durable responses from a single infusion
- ✓ Continuously surveil for and eliminate target cells
- ✓ Independent on MHC restriction, allowing to kill tumor cells with immune escape

CAR-T cell therapy, originally developed for hematologic malignancy, is showing strong potential in autoimmune diseases

B Cells targeted CAR-T Cell therapies are delivering unprecedented outcomes in hematological malignancies, and emerging as impactful new treatments modality for autoimmune diseases.

Pipeline



Diversified, Comprehensive and Balanced Product Pipeline Developed with a Forward-Looking, De-risked Development Approach

	Program ⁽¹⁾	Target	Indications ⁽²⁾ (Line of Treatment) ⁽³⁾	Pre-clinical	Phase I	Phase II/Phase III	BLA	Current Status / Expected Next Milestone	Commercial Rights
Cell Therapy	ex vivo CAR-T								
	Eque-cel (CT103A) 	BCMA	r/r MM (≥4L)	BLA approval 				BLA approved by NMPA in Jun 2023, ISAF, Macau SAR in Mar 2025, and DOH, Hong Kong in Nov 2025	Global ⁽⁴⁾
				BLA application accepted or submitted 				BLA accepted by HSA, Singapore in Jan 2025, BLA accepted by SFDA, Saudi in Jul 2025, and BLA dossier submitted with EDE, UAE in Sep 2025	
				IND (Japan)				Received IND approval in Oct 2025	
			r/r MM (2L/3L)	Phase III registrational trial				Phase III patient enrollment to be completed in 2H 2026	
			NDMM (High-risk 1L)	IIT				IIT to be completed in Q4 2025	
			MG	IND				Phase I patient enrollment expected to initiate in 1H 2026	
				IND (USA)				Phase I patient enrollment expected to initiate in 1H 2026	
			MS/SLE/IIM, etc.	IIT				IIT patient enrollment to be completed in Q4 2025	
			MS	IND (USA)				Phase I Patient enrollment expected to initiate in 2H 2026	
			SLE/LN	IND (USA)				Phase I Patient enrollment expected to initiate in 2027	
	IASO120 ⁽⁵⁾	CD19/CD22	r/r B-NHL (≥3L)	Phase II registrational trial				Phase II patient enrollment expected to initiate in 2H 2026	
	IASO118 ⁽⁵⁾	GPRC5D	r/r MM (≥4L)	IND				Phase I Patient enrollment expected to initiate in 2H 2026	
	IASO140 ⁽⁵⁾	BCMA/GPRC5D	r/r MM (≥4L)	IIT				IIT patient enrollment to be completed in Q4 2025	
	in vivo CAR-T								
IASO206	BCMA	r/r MM (≥4L)	IIT				IIT patient enrollment expected to initiate in 1H 2026 Expected to submit IND applications to US FDA and NMPA in 1H 2027	Global	
IASO207	CD19	B-NHL (≥3L) and SLE	IIT				IIT patient enrollment expected to initiate in Q4 2025 Expected to submit IND applications to US FDA and NMPA in 2H 2026		
IASO208	CD20	B-NHL (≥3L)	IIT				IIT patient enrollment expected to initiate in Q4 2025 Expected to submit IND applications to US FDA and NMPA in 2H 2026		
IASO209	Undisclosed	Undisclosed	Pre-clinical				Under development		
IASO210	Undisclosed	Undisclosed	Pre-clinical				Under development		
Antibody	Antibody								
	IASO782	CD19	ITP and wAIHA	Phase I clinical trial				Phase Ia patient enrollment expected to be completed in 2H 2026	Global
				IND (USA)				IND approved by US FDA in Jun 2023	
SLE			IND				Phase Ib patient enrollment expected to initiate in 1H 2027		

Abbreviations: BCMA = B cell maturation antigen; B-NHL = B-cell non-Hodgkin lymphoma; CAR-T = chimeric antigen receptor T cell; Eque-cel = Equecabtagene Autoleucel; IIM= idiopathic inflammatory myopathies; in vivo = within the living body; ITP = immune thrombocytopenia; LN = lupus nephritis; MG = myasthenia gravis; MS = multiple sclerosis; r/r MM = relapsed or refractory multiple myeloma; NDMM = newly diagnosed multiple myeloma; SLE = systemic lupus erythematosus; HM = hematologic malignancies; AD = autoimmune diseases; wAIHA = warm antibody autoimmune hemolytic anemia; BLA = Biologics License Application; U.S. FDA = U.S. Food and Drug Administration; IIT = investigator-initiated trial; IND = Investigational New Drug application; NCT = National Clinical Trial; NMPA = National Medical Products Administration of China.

★ Core Product ★ Commercialized Product 🌿 Orphan Drug Designation [---] Exempted by regulatory authority based on the clinical trials conducted in China

Notes:
1. Unless otherwise specified, the clinical trial was conducted in China.
2. The product or product candidate is intended to be used as monotherapy.
3. Treatment guidelines do not categorize the therapeutic approach for autoimmune diseases by lines of treatment. Industry convention defines patient population by disease subset, activity/severity and key prior-exposure classes.

4. Eque-cel had been granted Orphan Drug Designation for the treatment of r/r MM in Saudi Arabia and Korea.
5. IASO120, IASO118 and IASO140 were originally known as RD120, RD118 and RD140, respectively.

Overview of Senior Management

A Visionary Management Team with World-class Experts, Industry Veterans and Seasoned Entrepreneurs



Zhang Jinhua

*Founder, Executive Director,
Chairwoman and Chief
Executive Officer*

- Worked in investment banks and leading securities law firms, during which time she led the overseas and domestic listing and M&A restructuring projects of many Chinese companies
- Extensive experience in IPOs, mergers and acquisitions, and investment in the biopharmaceutical field. **Invested as angel investor in cell therapy startups** and served as director
- Obtained her **bachelor's degree in laws, and master's degree in laws**, both majoring in economic law, from Dongbei University of Finance and Economics



Dr. Zhang Yongke

*Chief Scientific
Officer and
Senior Vice
President*



Zhang Hua

*Chief Commercial
Officer and
Senior Vice
President*



Dr. Chen Jie

*Chief Medical Officer
and Senior Vice
President*



Dr. Hu Guang

*Vice President of
R&D*



Dr. Ren Chunli

*Senior Vice
President of
Registration*



Zhang Xinyu

*Senior Vice President
and Financial
Controller*



Growth Strategy



A Global Strategy from R&D to Commercialization



China-US R&D Center

- Established R&D center in US, R&D base in Nanjing, and global R&D center in Shanghai
- China's global leadership in CAR-T innovation

- ✓ China has established a favorable environment for CAR-T cell therapy development. IIT data may later support IND submissions, which enables early, lower-cost clinical exploration and accelerates the transition from research to registration
- ✓ NMPA and CDE provide accelerated regulatory pathways for CGTs — such as priority review, conditional approval, and breakthrough therapy designation — further shortening development timelines and facilitating commercialization



Pursuing Registration in International Markets

Registration strategy combines direct registration with bridging and multi-center studies:



Direct registration: Macau, Hong Kong, Singapore, Saudi Arabia, UAE, Malaysia, and South Korea

Bridging and multi-center studies: Japan and Brazil



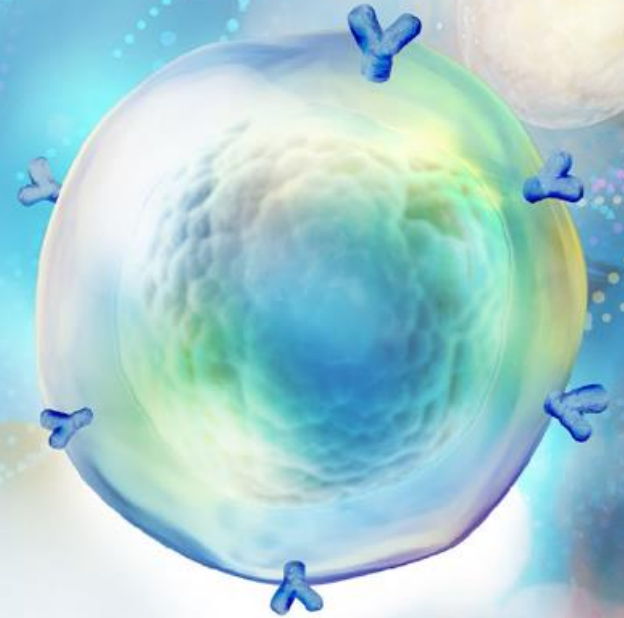
Global Collaboration

**“Products out, Patients in”
Global Strategy**

- **Multi-layer global treatment platform:** promoting early international access through NPPs and patient-referral programs
- **Strengthen BD and licensing activities:** co-development, out-licensing and in-licensing opportunities
- **Globally integrated, innovation-driven cell therapy provider:** monetize integrated platform through comprehensive, exportable partnership model encompassing process development, technology transfer, and GMP manufacturing support

SECTION 2

Global Commercialization



Leading Commercialization Capabilities in China and Early Success in Overseas Commercial Expansion

A Proven Commercialization Platform and Expanding Global Footprint Demonstrating Strong Execution Capability and International Competitiveness

IASO has built one of the most comprehensive commercialization platforms for ATMPs in China

Following the NMPA approval of Fucaso® (eque-cel) in 2023, we became one of the first Chinese biopharmaceutical companies to **independently commercialize** a domestically developed CAR-T cell therapy

Scientific innovation

Market success

In-house commercial organization / integration across the full commercialization cycle



The rapid rollout of Fucaso® sets an industry benchmark for commercial execution in China's ATMP sector

- Within one year of launch, Fucaso® achieved nationwide presence across **over 100** leading hospitals
- In 2024, the revenue of Fucaso® reached RMB191.8 million and its market share reached near **70%** in the China CAR-T cell therapy market for treatment of MM
- We have built a domestic sales network covering over **95 DTPs**, **21 pharmaceutical distribution companies** and over **180 treatment centers** located in over **40 cities** in China

Improving patient access and affordability while maintaining high treatment quality and compliance

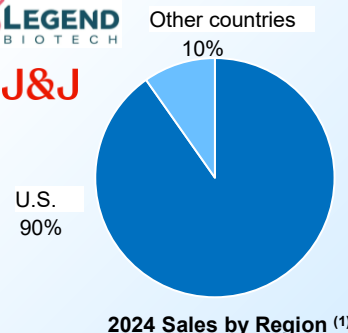
- Eque-cel was covered by **40** commercial insurances and Huimin insurance of **142** regions in China
- In December 2025, eque-cel was successfully included in the **National Commercial Insurance Innovative Drug Catalogue**

(Our success in China's complex healthcare environment) + (Growing recognition by global partners)

Positions IASO Bio as one of the few companies capable of delivering end-to-end value - from discovery and development to large-scale commercialization and international patient access

IASO has adopted an overseas expansion path distinct from other Chinese CAR-T companies

CARVYKTI



Approved for marketing in the U.S. by the FDA in February 2022



Approved for marketing in the EU by the EMA in May 2022



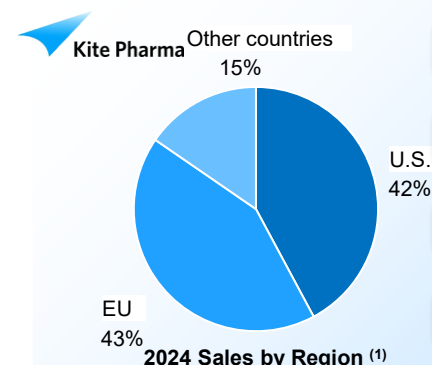
Approved for marketing in Japan by the PMDA in September 2022



Approved for marketing in China by the NMPA in August 2024



YESCARTA



Approved for marketing in the U.S. by the FDA in October 2017



Approved for marketing in the EU by the EMA in August 2018



Approved for marketing in Japan by the PMDA in January 2021



Approved for marketing in China by the NMPA in June 2021



Effectively Advance the Clinical Development and Registration of Eque-cel Globally

To Further Unlock the Clinical and Commercial Value of Our Core Product

Our international registration strategy combines direct registration with bridging and multi-center studies. Building upon our CMC capabilities, we are leveraging our clinical data to date to support registration with authorities in Macau, Hong Kong, Singapore, Saudi Arabia, UAE, Malaysia, and South Korea, while pursuing market approvals in Japan and Brazil via bridging studies to facilitate commercialization of eque-cel globally



US: From April to August 2024, we received IND approval from the U.S. FDA to conduct clinical trials of eque-cel in patients with MG, MS, and SLE/LN



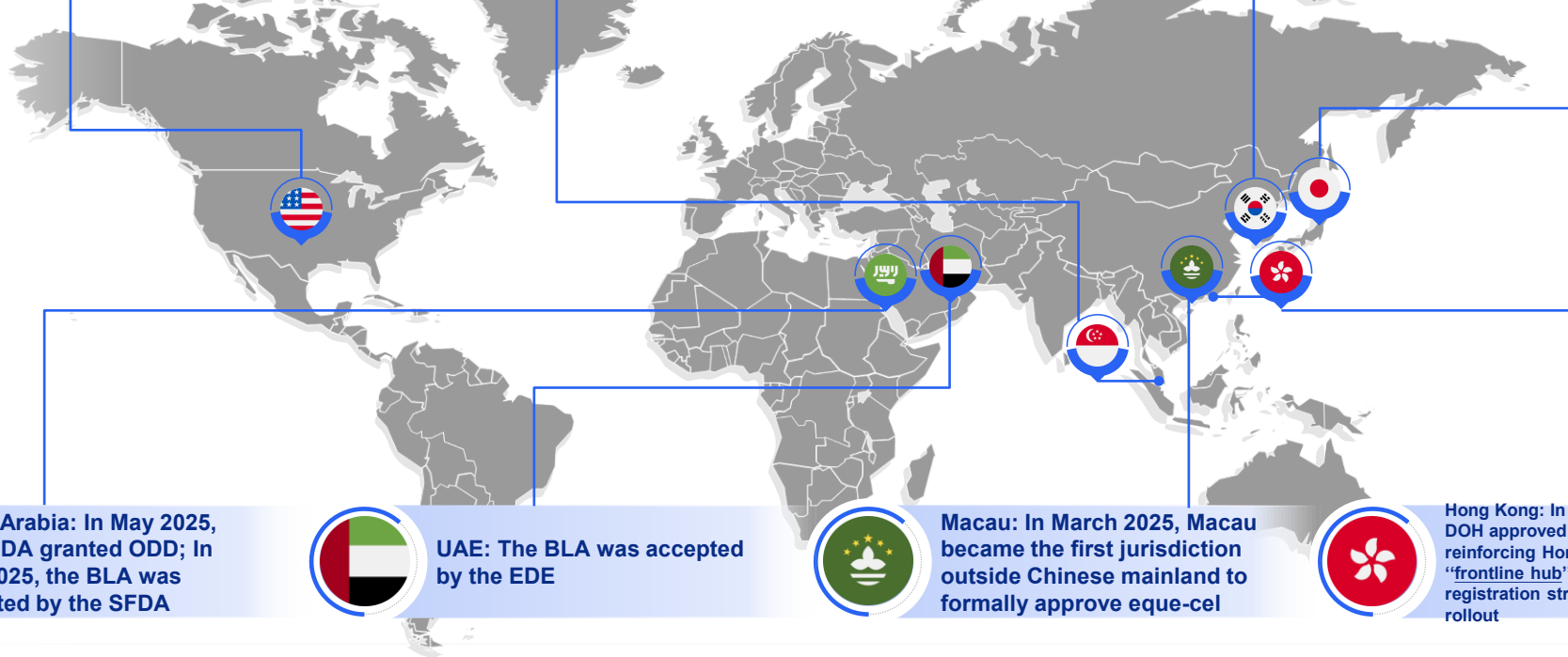
Singapore: In January 2025, the HSA accepted the BLA submitted by us



South Korea: In July and August 2025, eque-cel was granted ODD and Priority Review Designation, respectively, by the MFDS



Japan: In October 2025, we received IND approval from the PMDA



Saudi Arabia: In May 2025, the SFDA granted ODD; In July 2025, the BLA was accepted by the SFDA



UAE: The BLA was accepted by the EDE



Macau: In March 2025, Macau became the first jurisdiction outside Chinese mainland to formally approve eque-cel



Hong Kong: In November 2025, the DOH approved the BLA for eque-cel, reinforcing Hong Kong's role as a "frontline hub" in our localized registration strategy and international rollout

Multi-Pronged Global Commercialization Strategy

Early Success in Extending Commercialization Model Globally, Expanding Global Patient Access

- Our PIC/S-compliant GMP manufacturing system, combined with global regulatory expertise and established comparability frameworks, provides a robust foundation for multi-regional supply, technology transfer, and co-commercialization
- Building upon our multi-layer global treatment platform, we are promoting early international access through NPPs and patient-referral programs, while advancing marketing approvals and insurance inclusion in selected regions

Multi-regional Supply

Our “manufactured in China, supplied globally” model designed to deliver advanced CAR-T cell therapies efficiently to international markets

The end-to-end manufacturing and CMC capability provides the infrastructure necessary for global expansion, enabling regional manufacturing, cross-border technology transfer, and multi-regional clinical trial supply under harmonized quality standards

Integrated CAR-T Solutions

We plan to monetize our integrated platform through our “China Solution” — a comprehensive, exportable partnership model encompassing process development, technology transfer, and GMP manufacturing support

We offer overseas partners integrated full cycle solutions featuring all-in-one automated streamed cell processing equipment, plasmid, lentiviral vector, consumables together with technology transfer and related services

Co-commercialization

Our active business development engagements with multinational pharmaceutical companies, regional healthcare groups, and academic centers have led to ongoing collaborations and partnership discussions in licensing, co-development, and regional manufacturing, further validating the global competitiveness of our platform and products



Patient Referral Programs

We have established a comprehensive patient journey management system, enabling efficient overseas patient referral and treatment service delivery

Through the patient referral/medical tourism program, patients from overseas are treated with eque-cel at certified treatment centers in China, with 40+ orders placed under this program

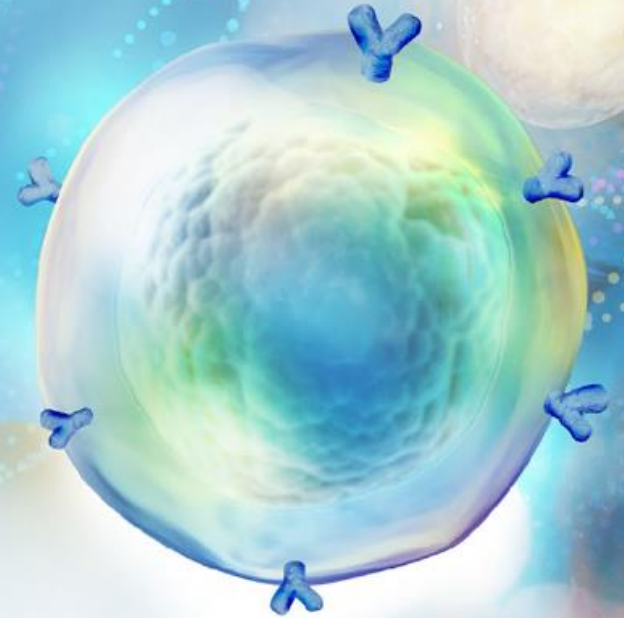
Named Patient Program (NPP)

We are the first Chinese company to realize cross-border supply of domestic CAR-T cell therapy via the NPP in Hong Kong, with multiple successful cases completed to date

Given that most regions globally still lack commercialized BCMA CAR-T treatment access, advancing the NPP first in these markets is a critical part of our international presence

SECTION 3

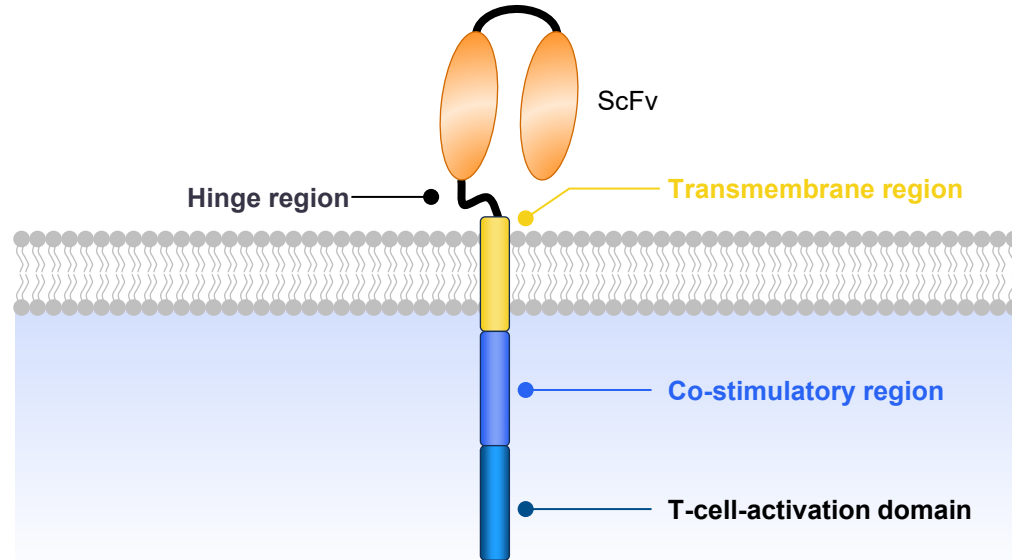
Pipeline Overview



Eque-cel - First-in-class and Potentially Best-in-class BCMA CAR-T Therapy

Overview

Mechanism of Action and Competitive Advantages



The World's First Fully Human CAR-T Cell Therapy
The first BCMA-targeted CAR-T therapy approved in China

Using a fully human scFv targeting BCMA with much lower immunogenicity

- The CAR is constructed by fusing an extracellular binding domain of a fully human scFv that targets BCMA, with intracellular signaling domains (consisting of 4-1BB costimulatory and CD3- ζ activation domains) that enable T cell activation and persistence
- After infusion, the CAR on the engineered T cells bind to BCMA on malignant plasma cells, leading to potent cytotoxic effects with cytokine release

Competitive Advantages

-  **Optimized CAR Structural Design, Fully Human CAR Structure with Optimal and Consistent Binding to BCMA**
-  **Superior Safety and Efficacy Profile Compared to Other BCMA-targeted CAR-T Cell Therapies**
-  **Robust and Self-Reliant Manufacturing Capabilities, with the Ability to Independently Produce the Lentiviral Vector**
-  **Comprehensive Global Registration and Development Strategy, Leveraging China's Manufacturing Strengths While Advancing Localized Regulatory and Market Access Strategies Worldwide**
-  **Comprehensive Commercialization Strategy, with One of the Most Comprehensive Commercialization Platforms Built for ATMPs in China, Extending Our Commercialization Model Globally**
-  **Comprehensive Indication Expansion Plan, Promising Efficacy in Other Indications Like Autoimmune Diseases**

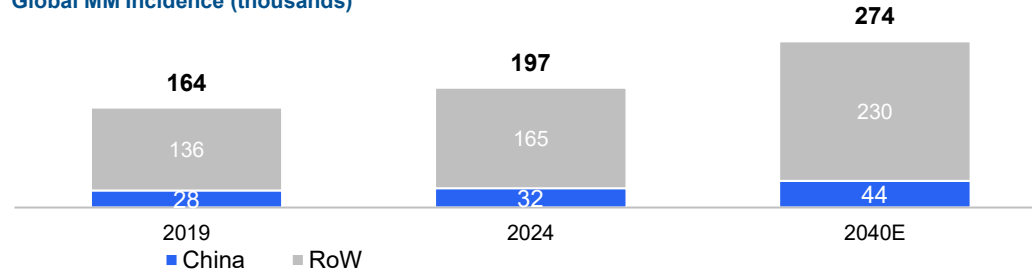
Eque-cel - First-in-class and Potentially Best-in-class BCMA CAR-T Therapy (Cont'd)

Market Opportunities and Competition

MM

- MM is a cancer of plasma cells in the bone marrow. It develops when healthy plasma cells become malignant and proliferate uncontrollably. The disease often leads to significant skeletal damage, including osteolytic lesions, osteopenia, and pathological fractures
- CAR-T cell therapies targeting B cell-associated antigens, including BCMA, have emerged as a critical strategy for the treatment of MM. BCMA directed CAR-T cell therapies have demonstrated deep and durable responses in heavily pretreated relapsed or refractory patients, leading to regulatory approvals in the U.S., the EU, and China

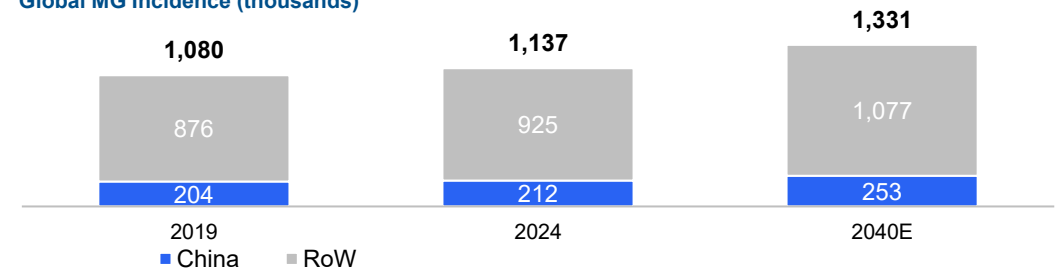
Global MM Incidence (thousands)



MG

- MG is an autoimmune neurological disorder characterized by defective transmission at the neuromuscular junction. It is considered a classic example of an antibody-mediated autoimmune disease
- Globally, treatment options for MG remain limited, particularly for patients with generalized or refractory disease. BCMA-targeted CAR-T cell therapy offers a potential treatment for MG by eliminating B cells and plasma cells that produce pathogenic autoantibodies against AChR or MuSK

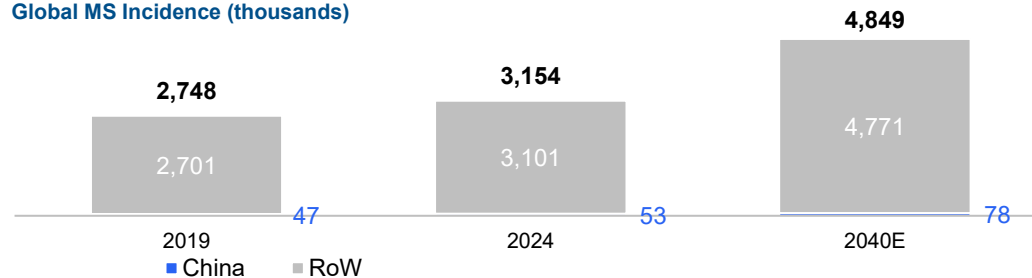
Global MG Incidence (thousands)



MS

- MS is an immune-mediated inflammatory disease of the CNS that causes demyelinating lesions dispersed in time and space
- Although DMTs have transformed outcomes for many patients, durable neuro protection and reliable prevention of disability progression remain elusive. Significant unmet needs persist in the management of MS worldwide

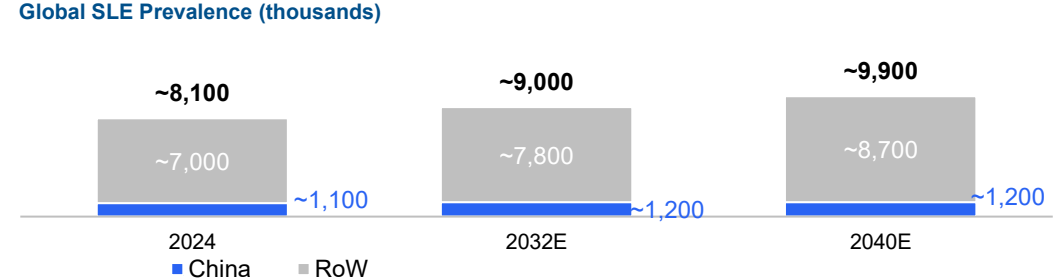
Global MS Incidence (thousands)



SLE

- SLE is a multisystem autoimmune disease in which the immune system attacks the body, leading to symptoms such as rash, joint pain and swelling, fever, chest pain, hair loss, mouth ulcers, swollen lymph nodes, and general debilitation
- BCMA is highly expressed on plasma cells, particularly the CD19-CD138+ subset that produces pathological autoantibodies in SLE, offering a novel therapeutic approach for SLE

Global SLE Prevalence (thousands)



Eque-cel - First-in-class and Potentially Best-in-class BCMA CAR-T Therapy (Cont'd)

Clinical Data Comparisons

Demonstrated clear advantages over other CAR-T cell therapies in multiple key measures

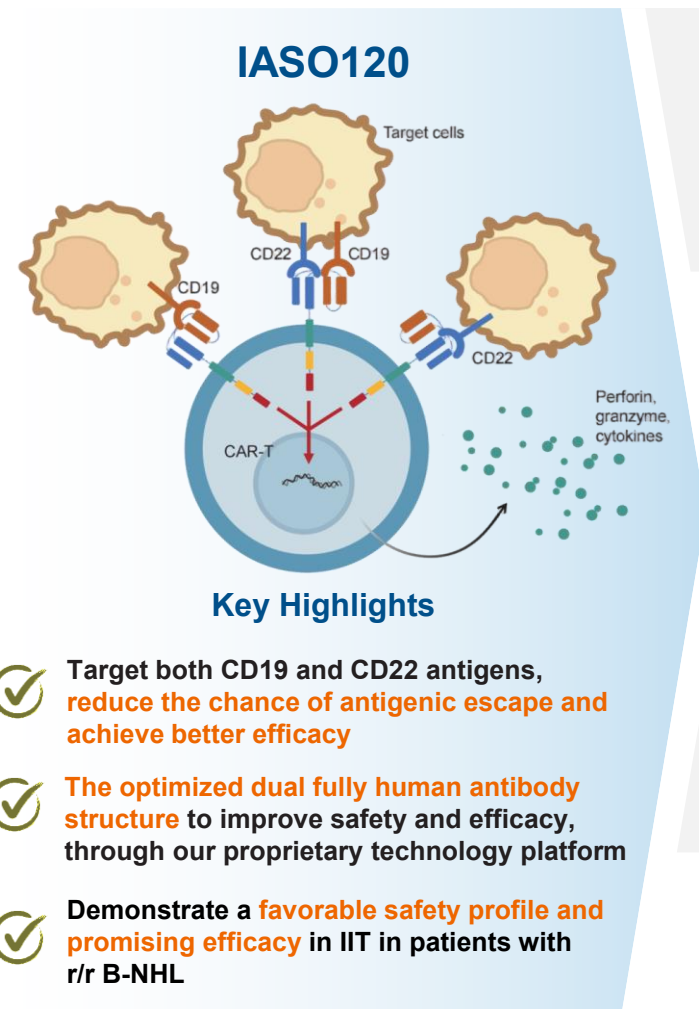
	Fucaso®		Abecma®	Carvykti®	赛恺泽® (Zevor-cel)
	Patients without prior CAR-T N=97	All patients N=109	N=128	N=97	N=102
Approval	China Jun. 2023		USA Mar. 2021	USA Feb. 2022	China Feb. 2024
Overall Response	98.9%	96.3%	73%	97.9%	92.2%
≥CR	88.4%	83.2%	33%	82.5%	71.6%
mPFS (months)	35.9	30.5	8.8	34.9	/
12-month PFS	85.5%	80.0%	/	/	76.3%
CRS Grade ≥3	1.0%	0.9%	5.5%	5.1%	6.9%
CAR-T-associated Neurotoxicity (Including ICANS) ≥ Grade 3	0%		4%	11%	0%
Parkinsonism	0%		/	5%	0%
MDS Incidence Rate	0%		2/137	7/97	0%
AML Incidence Rate	0%		0%	3/97	0%

Eque-cel - First-in-class and Potentially Best-in-class BCMA CAR-T Therapy (Cont'd)

Broad Indication Potential and Major Clinical Development Plan

≥4L r/r MM	• We secured conditional BLA approval from the NMPA in June 2023, and submitted the application for full marketing approval in October 2025 • We have submitted an IND application for conducting a pivotal clinical trial to the PMDA in September 2025, and we obtained approval in October 2025. We expect to complete the trial in the first half of 2027 and submit the BLA based on the collected data in 2028
2L/3L r/r MM	• We expect to complete patient enrollment for the ongoing Phase III clinical trial in China in the second half of 2026. Based on the data collected from the Phase III clinical trial in China, we plan to submit BLA in 2028, with approval expected in 2029 • Furthermore, we plan to submit an IND application to the PMDA in the first half of 2026, initiate the trial in the second half of 2026, and complete patient enrollment in the second half of 2027
High Risk NDMM	• The ongoing IIT study suggested that eque-cel could be safely and effectively used in first-line treatment: all NDMM patients responded to treatment, resulting in an ORR of 100%, with 93.8% achieving sCR or CR • The IIT study is expected to be completed in the fourth quarter of 2025
MG	• Eque-cel showed compelling efficacy in two refractory MG patients with distinct autoantibody profiles according to the IIT: a 33-year-old woman with AChR and Titin-IgG-positive disease achieved both functional and serological remission, with restoration of strength, discontinuation of symptomatic therapy, and sustained remission through 21 months, including a successful pregnancy and delivery; a 60-year-old woman with MuSK-IgG4-positive MG showed rapid functional recovery • We plan to initiate patient enrollment for a Phase I clinical trial of eque-cel in China in the first half of 2026, and simultaneously in the U.S.
MS	• According to the IIT, in progressive MS, eque-cel demonstrated rapid expansion and persistence of CAR T cells, with significant B-cell depletion observed by day 10 • In the U.S., we plan to initiate Phase I clinical trials of eque-cel for MS in the second half of 2026
IMNM	• According to the IIT, in a patient with IMNM, eque-cel produced durable clinical benefit. Following treatment, the patient experienced only mild transient CRS and went on to demonstrate near-complete neurological recovery, regaining independent mobility and muscle function by nine months

IASO120 – Potential First-in-class, Fully Human CD19/CD22 Dual-targeted CAR-T Cell Therapy Candidate



Drug Design and MoA

- A potential first-in-class, internally developed, fully human, dual-targeted CAR-T therapy targets both CD19 and CD22, aiming to overcome the limitations of existing CAR-T therapies that target only one antigen
- Novel bispecific CAR construct aims to overcome challenges like antigen escaping mediated relapse and potentially provide deeper and longer-lasting therapeutic responses

Market Potential ⁽¹⁾

- Lymphomas are hematologic malignancies originating from the lymphoid system, with the two main categories being Hodgkin lymphoma and NHL. DLBCL being the most prevalent subtype
- Significant unmet medical needs remain in the treatment of DLBCL. Treatment options for relapsed or refractory DLBCL are limited. This highlights the urgent need for safer, more tolerable treatments that preserve efficacy while minimizing adverse effects
- IASO120 stands out as the most clinically advanced bispecific CAR-T cell therapy for DLBCL in China

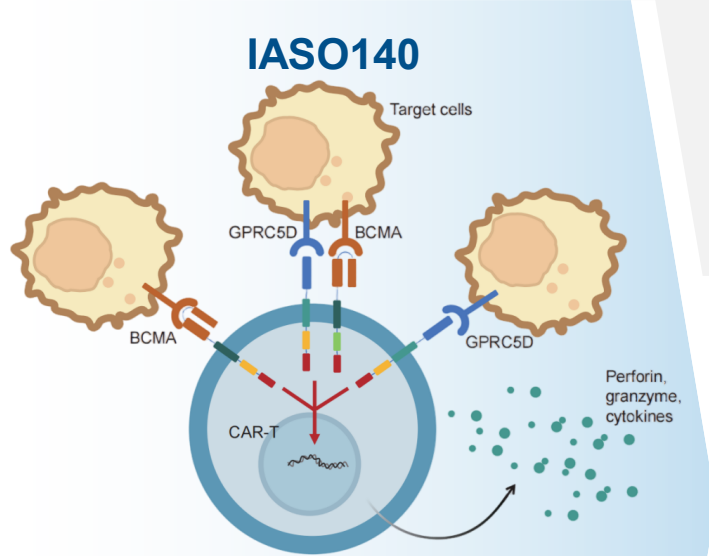
Clinical Data

- In an IIT in patients with r/r B-NHL, IASO120 demonstrated favorable safety profile and promising efficacy
- Best ORR of 65.2%. Long-term follow-up showed durable responses, with only 2 of 13 CR patients experiencing relapse at 11.3 and 23.95 months, and a median time to best response of 2.83 months
- No Grade 5 AEs reported. Most CRS cases were Grade 1-2 (82.6%), with only one patient experiencing Grade 3 CRS; all events were reversible, and no ICANS cases were observed

Development Plan

- We are conducting a Phase I/II clinical trial for r/r B-NHL
- Plan to initiate the Phase II study in 2H 2026, upon completion of commercial-scale manufacturing process validation

IASO140 – BCMA/GPRC5D Dual-targeted CAR-T Cell Therapy



Key Highlights

- ✓ **High expression of both BCMA and GPRC5D-targeting CARs**, at levels comparable to their respective single-targeting CAR-T cells
- ✓ **High efficacy based on preclinical studies**
- ✓ **Fast manufacturing process** of CAR-T cells requires only 3-4 days to complete, benefiting patients in need of timely therapy

Drug Design and MoA

- **IASO140 simultaneously targets both BCMA and GPRC5D**, addressing the challenge of antigen escape seen with BCMA-targeted treatments for r/r MM
- **Utilize a bicistronic construct**, enabling CAR-T cells to independently recognize and eliminate tumor cells expressing either target, improving the likelihood of complete tumor eradication despite antigen heterogeneity
- **Developed using the proprietary FAST CAR-T platform**, IASO140 benefits from rapid, efficient production, offering advantages in scalability, speed, and cost-effectiveness

Market Potential ⁽¹⁾

- Dual targeting of BCMA and GPRC5D offers complementary advantages for bispecific T cell therapies and mitigates antigen escape, maintaining cytotoxic pressure and overcoming tumor heterogeneity
- **2 CAR-T cell therapy candidates** targeting both BCMA and GPRC5D had entered clinical development globally, one of which was being developed in China

Clinical Data

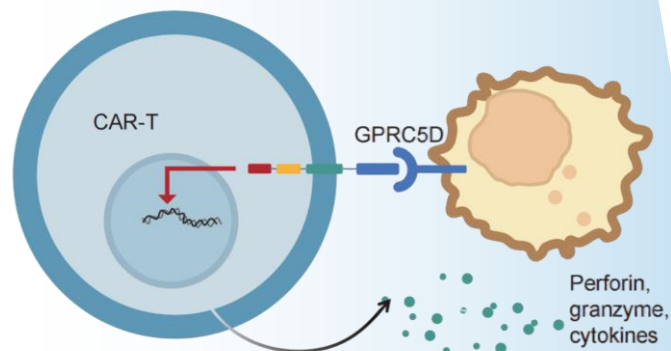
- **The functional performance of IASO140 was shown to be equivalent to single-targeting CAR-T cells across both in vitro and in vivo models**
- **In cytotoxicity assays**, IASO140 exhibited strong activity against tumor cells expressing either BCMA or GPRC5D, accompanied by robust cytokine secretion and proliferative capacity
- **In animal studies**, we observed consistent results, where IASO140 provided effective tumor control on par with single-target CAR-Ts

Development Plan

- Patient enrollment of the ongoing IIT of IASO140 for r/r MM is expected to be completed **in Q4 2025**
- Plan to submit IND application to NMPA **in 2H 2027**

IASO118 – GPRC5D-targeted Fully Human CAR-T Cell Therapy Candidate

IASO118



Key Highlights

- ✓ Derived from our proprietary fully human single-domain antibody library, offering advantages of high affinity, high specificity, and low immunogenicity
- ✓ Identified through our proprietary technology platform, showing optimal binding affinity to GPRC5D
- ✓ Enhanced antitumor activity compared with benchmark CAR-T products

Drug Design and MoA

- A fully human CAR-T cell therapy candidate targeting GPRC5D, which is designed for MM patients who have not responded to BCMA-targeted CAR-T therapy or those who have experienced antigen escape, potentially as sequential or combination strategies
- GPRC5D shows high expression on malignant plasma cells of patients, offering a favorable therapeutic window and being an ideal target for immune effector cell-based therapies.
- IASO118 leverages T cells genetically modified to express a CAR that specifically recognizes GPRC5D, which suggests the potential to overcome resistance mechanisms such as antigen loss and to be used in combination with BCMA-targeting agents

Market Potential ⁽¹⁾

- GPRC5D has recently emerged as a promising alternative target, which is expressed independently of BCMA. The median positivity rate of GPRC5D in newly diagnosed MM patients is 32.68%.
- Globally, 6 candidates targeting GPRC5D are under clinical development, with the most advanced product in the U.S. having reached Phase III. In China, 3 candidates are in development, with the most advanced product currently in Phase I/II

Clinical Data

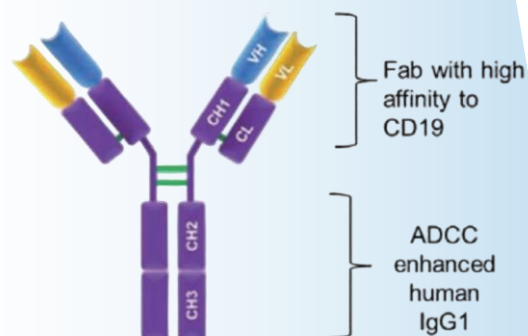
- The IIT of IASO118 showed promising results in patients with MM, including those who have received prior BCMA CAR-T cell therapy
- Efficacy: a high ORR of up to 94.4%, with a substantial proportion of patients achieving CR/sCR (72.2%) and MRD negativity (88.2%)
- Safety: although CRS occurred in the majority of patients, most cases were mild (Grade 1), and all resolved after treatment. ICANS was rare, with only one case observed, which also resolved
- The relevant research findings have been published in Blood

Development Plan

- Obtain IND approval for Phase I clinical trial from NMPA
- Plan to initiate patient enrollment for a Phase I clinical trial in 2H 2026

IASO782 – Anti-CD19 Monoclonal Antibody, A Potential First-in-class Therapy

IASO782



Key Highlights

- ✓ Fully human anti-CD19 monoclonal antibody with **low immunogenicity**
- ✓ **ADCC enhancement** to exhibit significantly improved target cell-killing activity
- ✓ Our CD19 sequence is protected by **long-term patent**
- ✓ **Superior preclinical efficacy**: Rapid and sustained depletion of B cells *in vitro* and *in vivo* preclinical studies

Drug Design and MoA

- A **fully human, anti-CD19 monoclonal antibody**, which is designed with the potential to target a broad spectrum of autoimmune conditions, such as ITP, wAIHA and SLE
- **IASO782 has been engineered at the Fc region to enhance ADCC**. Through both ADCC and ADCP, IASO782 depletes CD19+ B cells and plasma cells, thereby reducing the production of autoantibodies and addressing autoimmune diseases mediated by antibodies or B cells

Market Potential ⁽¹⁾

- ITP is an acquired autoimmune bleeding disorder with a steadily rising global. Current treatments often fail to achieve long-term remission, with **sustained responses in less than 60% of patients**, and **no therapy offers a true cure**
- wAIHA is the most common type of autoimmune hemolytic anemia, facing significant unmet medical need. **No therapies have been officially approved by the U.S. FDA or EMA**
- IASO782 is the **only CD19-targeted antibody therapy** under clinical development in China.

Clinical Data

- **In in vitro preclinical studies**, IASO782 has demonstrated superior efficacy compared with benchmark CD19-targeting therapies
 - High binding affinity to CD19 on the cell surface, which allows for more efficient and selective targeting of B cells
 - This enhanced binding may translate into improved B-cell clearance in vitro, suggesting a greater potential for eliminating pathogenic or malignant B cells
- **In in vivo preclinical studies using hCD19 humanized HE mice**, a single dose of IASO782 demonstrated rapid and sustained depletion of B cells
 - Following administration, B cell counts decreased sharply
 - This reduction was maintained for 14 days, reflecting the durable PD effect of IASO782 and its potential to provide effective B-cell targeting over an extended period

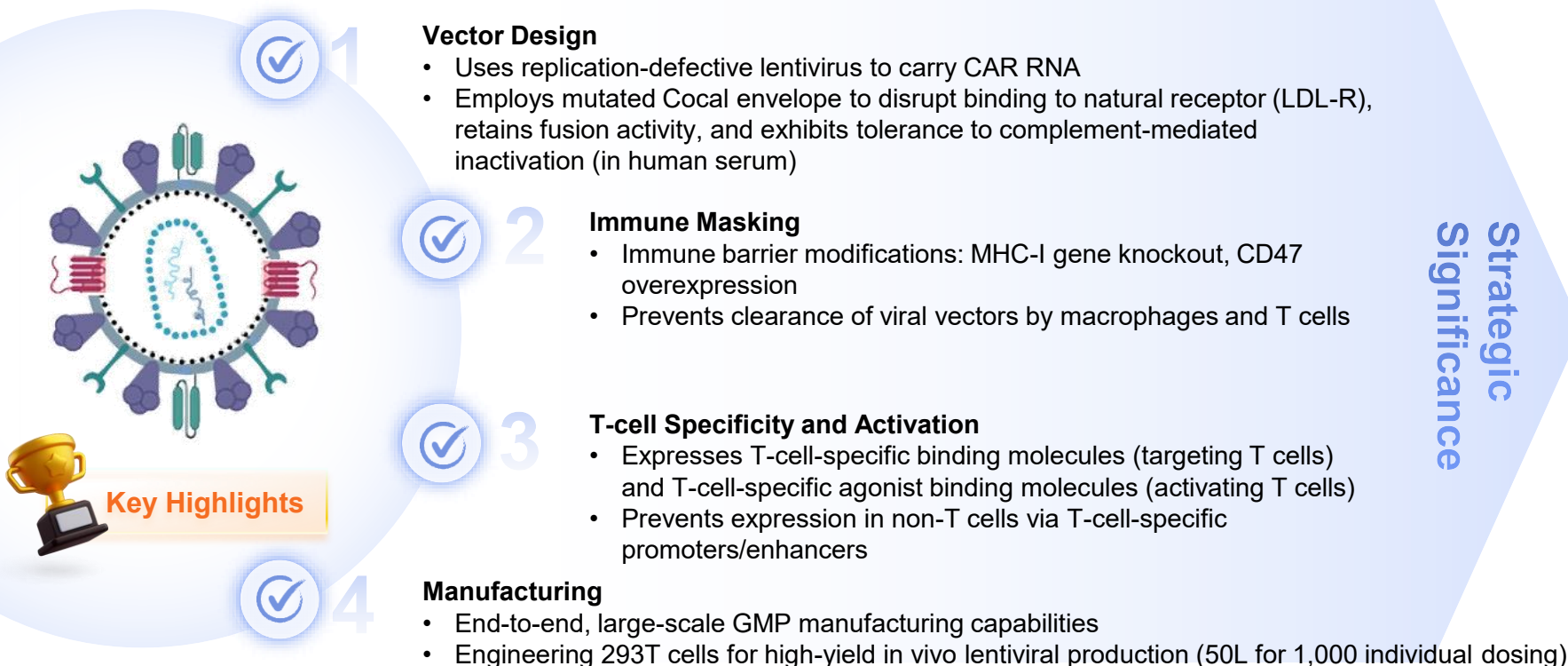
Development Plan

- Received IND approvals from both the NMPA and FDA to initiate clinical trials of ITP and wAIHA **in June 2023**
- Phase Ia patient enrollment for ITP and wAIHA expected to be completed **in 2H 2026**
- Phase Ib patient enrollment for SLE expected to initiate **in 1H 2027**

In vivo CAR-T Platform

Platform Technologies

- We are globally advancing *in vivo* CAR-T technologies that enable the direct generation of CAR-T cells within the patient's body, representing a paradigm shift in how cell therapies are discovered, manufactured, and delivered
- Our proprietary lentiviral vector engineering system incorporates a modified envelope glycoprotein and a T-cell-targeting ligand to achieve highly selective transduction of circulating T cells in vivo



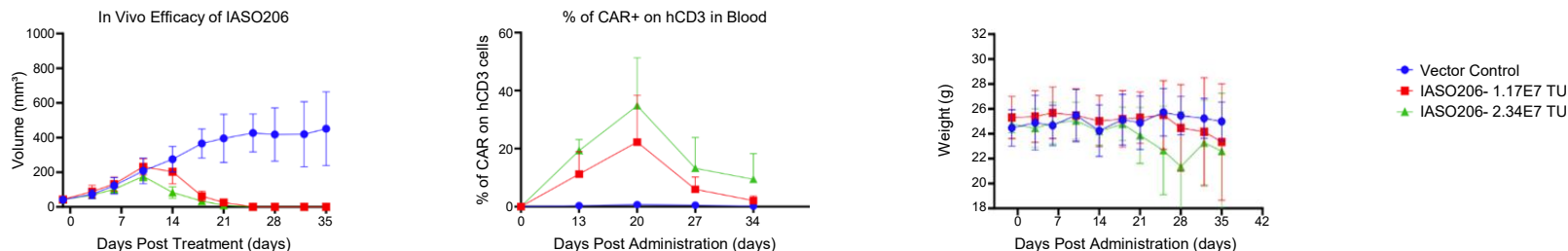
Extending our leadership in cell therapy innovation

Reinforcing our long-term competitive advantage as one of the few companies with the integrated expertise to develop, manufacture, and commercialize both ex vivo and in vivo CAR-T products

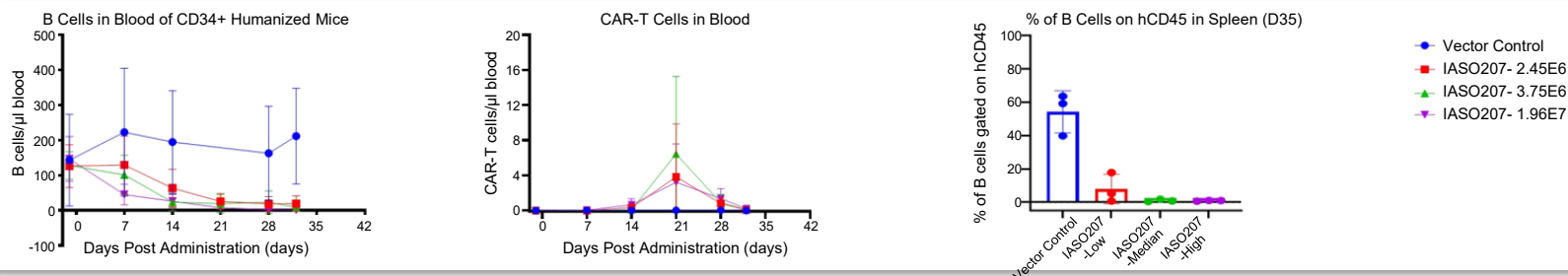
In vivo CAR-T Platform (Cont'd)

Pre-clinical Data

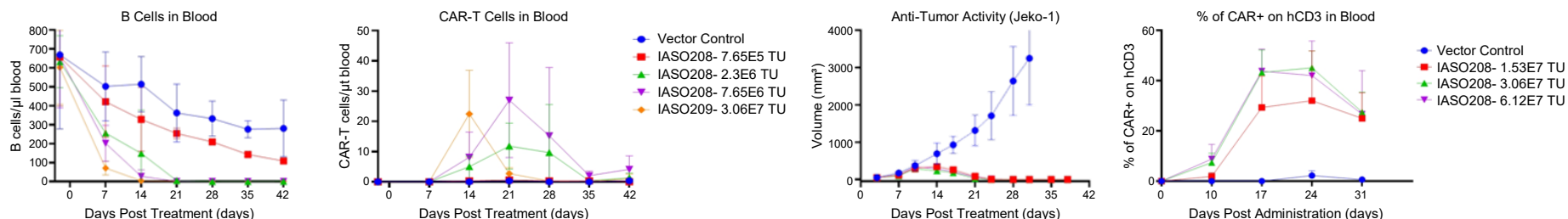
BCMA-Targeted In Vivo CAR-T Using Humanized VHH (IASO206)



CD19-Targeted In Vivo CAR-T Using Humanized scFv (IASO207)



CD20-Targeted In Vivo CAR-T Using Humanized VHH (IASO208)



In vivo CAR-T Platform (Cont'd)

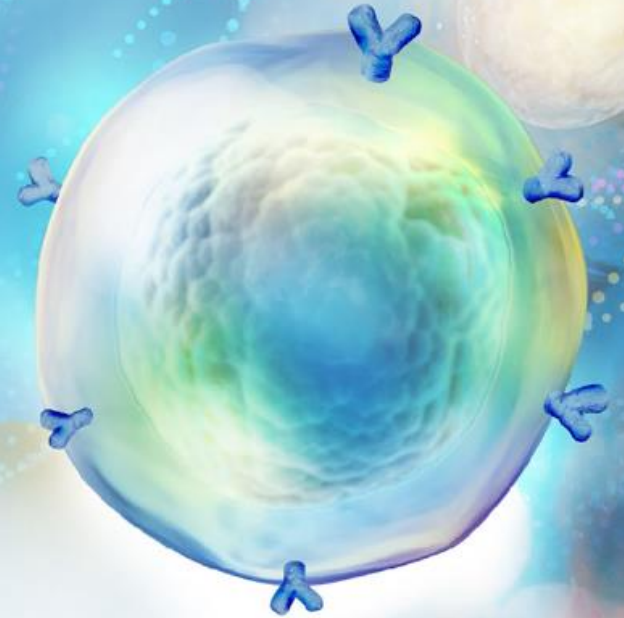
Pipeline

- We are developing a pipeline of *in vivo* CAR-T product candidates for the treatment of hematologic malignancies and autoimmune diseases
- These programs leverage our proprietary vector design to **achieve precise immune-cell targeting and controlled CAR expression**, with the goal of improving safety and therapeutic efficacy. The platform also offers **modular flexibility**, allowing the sequential or combined delivery of different CAR constructs to achieve multi-specific or tunable immune responses

Program	Target	Indications (Line of Treatment)	Pre-clinical	Phase I	Current Status / Expected Next Milestone
IASO206	BCMA	n/r MM (≥4L)	IIT		IIT patient enrollment expected to initiate in 1H 2026 Expected to submit IND applications to US FDA and NMPA in 1H 2027
IASO207	CD19	B-NHL (≥3L) and SLE	IIT		IIT patient enrollment expected to initiate in Q4 2025 Expected to submit IND applications to US FDA and NMPA in 2H 2026
IASO208	CD20	B-NHL (≥3L)	IIT		IIT patient enrollment expected to initiate in Q4 2025 Expected to submit IND applications to US FDA and NMPA in 2H 2026
IASO209	Undisclosed	Undisclosed	Pre-clinical		Under pre-clinical development
IASO210	Undisclosed	Undisclosed	Pre-clinical		Under pre-clinical development

SECTION 4

Manufacturing Capabilities



Robust and Self-Reliant Manufacturing Capabilities

The ability to independently produce the lentiviral vector used in eque-cel represents a core technological and strategic capability underpinning our rapid manufacturing turnaround, continuous process innovation, yield improvement, and global supply readiness

Manufacturing Facilities



Our commercial manufacturing site in Nanjing is a **PIC/S-certified facility for ATMP production** in China, designed and operated to meet the stringent standards of major global regulatory authorities such as the **FDA, EMA, and PMDA**



Ex vivo lentiviral vector

The proprietary lentiviral vector was engineered specifically for eque-cel, enabling **consistent transduction efficiency and stable CAR expression**. The integrated LVV-CAR-T manufacturing process developed for eque-cel **mitigates supply risks, reduces cost, and allows rapid manufacturing turnaround**



In vivo CAR-T products

Building upon our existing drug development platforms, we plan to continuously upgrade our technology platforms, including our *in vivo* platform and FAST CAR-T platform, to support the development of **next generation innovative drugs**



Production capacity expansion

Our facilities feature full-cycle production capabilities covering plasmids, viral vectors and CAR-T cells and in-house analytical and quality control capabilities, with a currently approved manufacturing capacity of **500** patients per year. We are planning to expand capacity to **1,000** patients within one year

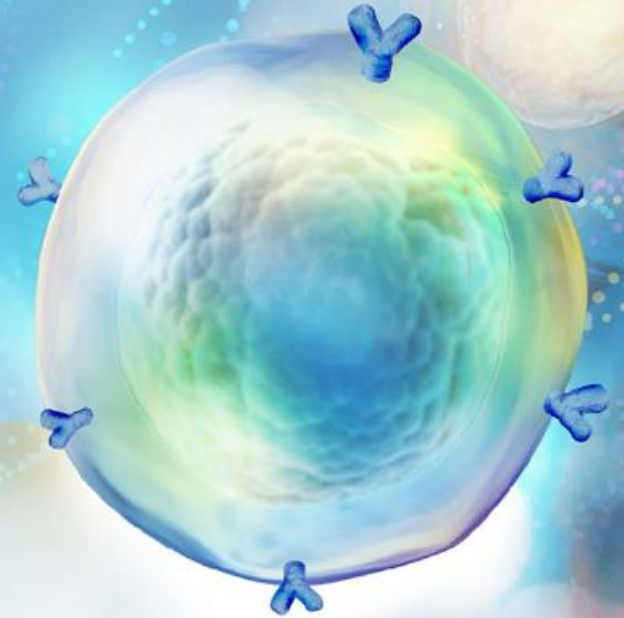


Cross- border supply

Our commercial manufacturing site is a PIC/S-certified facility for ATMP production, ensuring **global supply readiness**, and reinforcing our position as a fully integrated, **globally competitive** cell-therapy manufacturer

SECTION 5

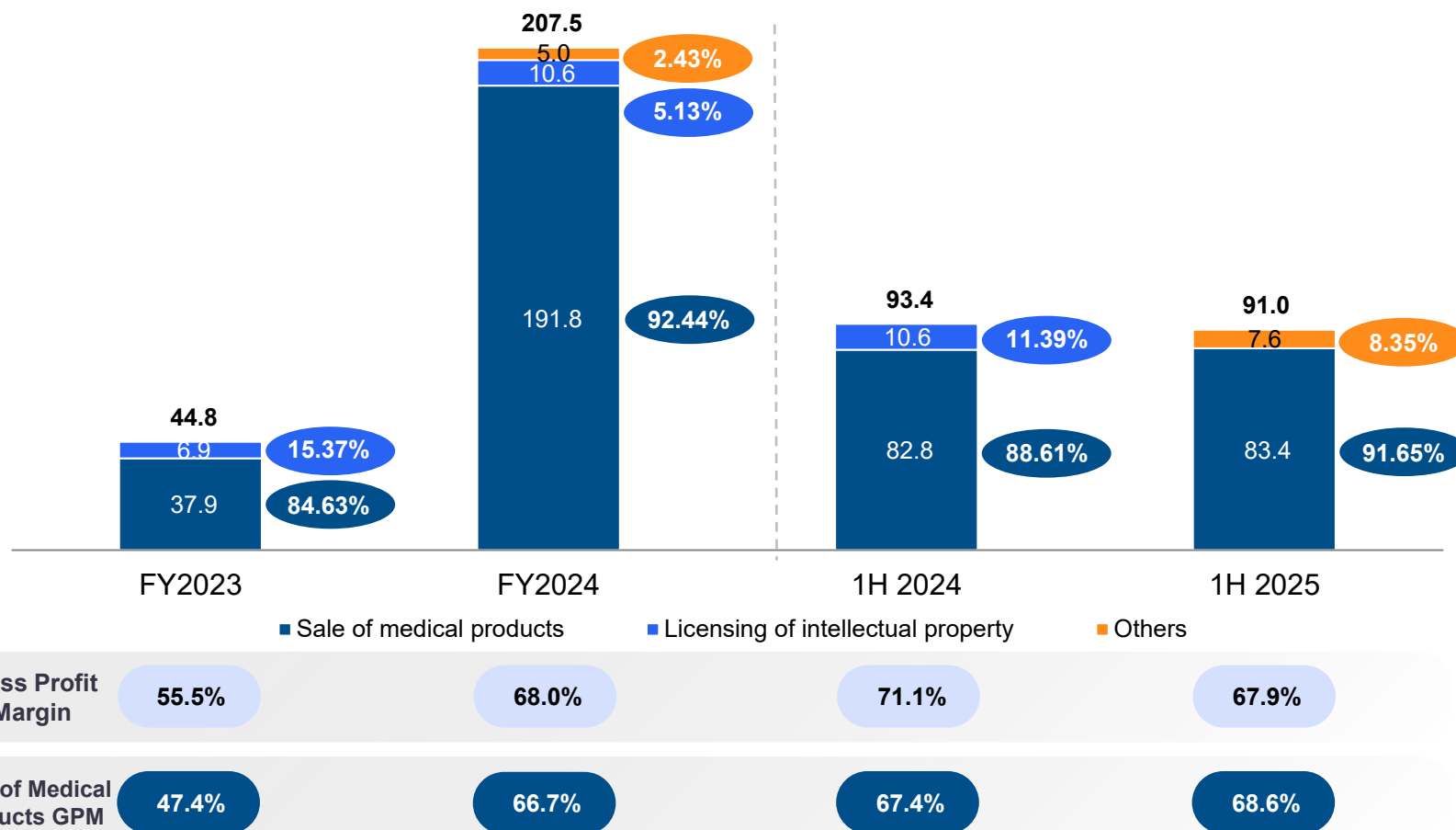
Financial Overview



Revenue and Gross Profit Margin

2023 - 1H 2025 Revenue Segment Breakdown and Gross Profit Margin

(in million RMB)



Revenue Analysis

- Our revenue primarily generated from sales of medical products after Fucaso obtaining conditional approval in China in June 2023, licensing of intellectual property, and CRO and other services
- By geographical markets, we derived a material portion of our revenue from sales through Chinese mainland, accounting for 84.6%, 91.6%, 88.6%, and 88.5% in the year ended December 31, 2023 and 2024, and the six months ended June 30, 2024 and 2025, respectively

Cash Flow and Cash Balance

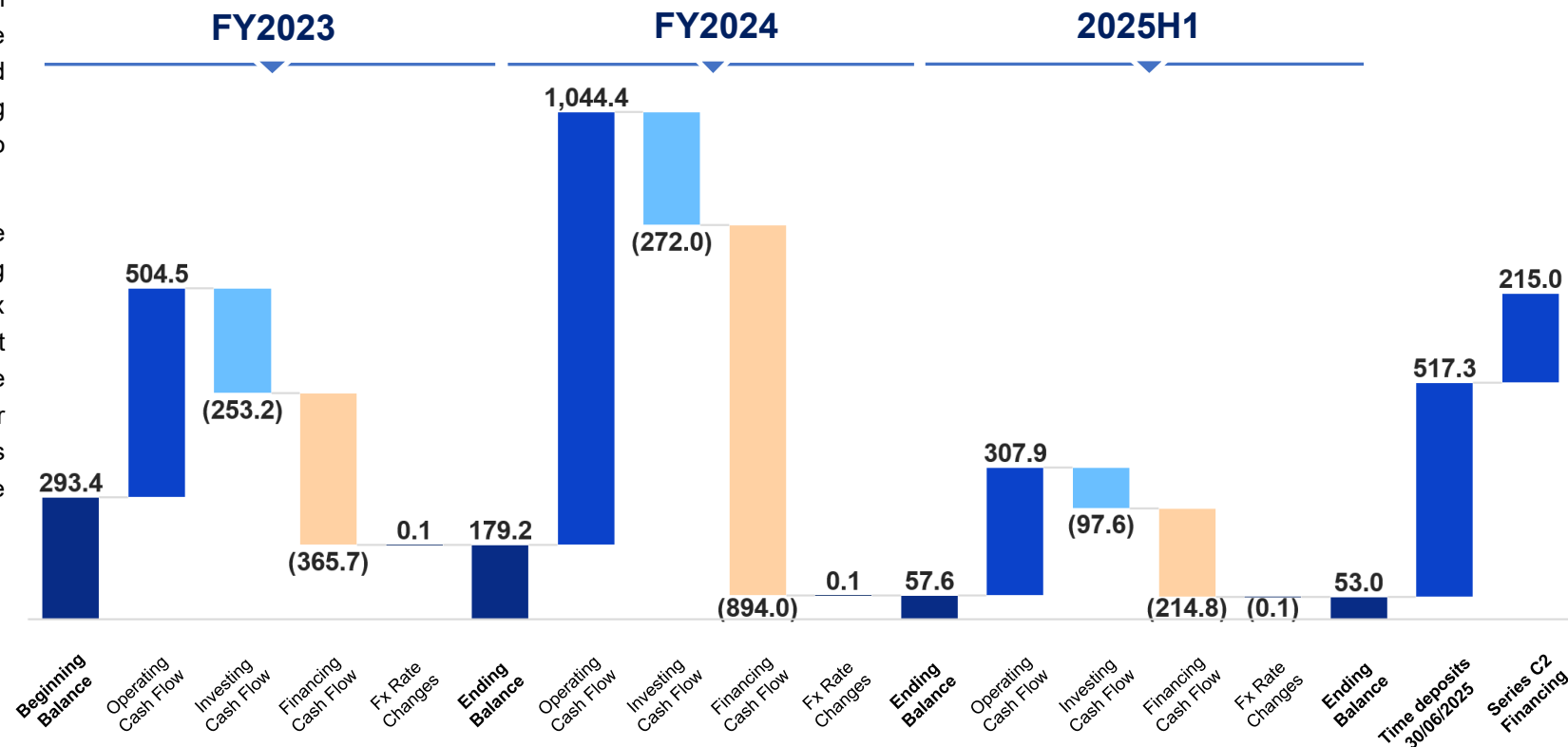
Cash Position

- Cash Position:** As of June 30, 2025, the Company held cash and cash equivalents of RMB 53.0 million, time deposits of **RMB 520 million**, and proceeds from the Series C2 financing in the second half of 2025 amounting to **RMB 220 million**
- Cash Burn Analysis:** Assuming the average monthly cash burn rate going forward of 1.1 times the level in the six months June 30, 2025, we estimate that our cash and cash equivalents and time deposit as of June 30, 2025, together with the net proceeds from the Series C2 financing, will be able to maintain the financial viability for over 2 years

> 2 Years

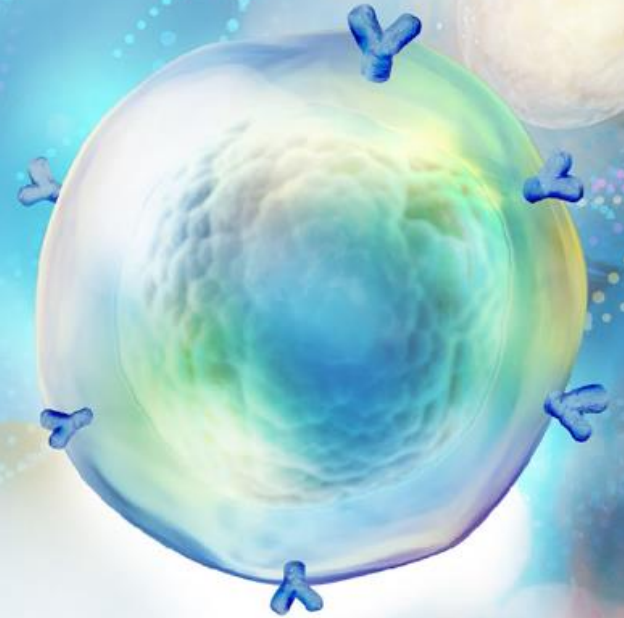
Cash Balance and Net Cash Flow

(RMB million)



SECTION 6

Conclusion



Potential Milestone

High-Quality Development of Pipeline Driven by Sustainable Momentum

Future Catalyst

- **CT103A**: To receive full marketing approval from NMPA of eque-cel for the treatment of r/r MM ($\geq 4L$); Phase I clinical trial of eque-cel for treatment of autoimmune diseases (including MG, MS and SLE/LN) to initiate in China and the U.S.
- **IASO120**: For the indication r/r B-NHL ($\geq 3L$), Phase II patient enrollment to initiate in China
- **IASO118**: For the indication r/r MM ($\geq 4L$), Phase I patient enrollment to initiate in China
- **IASO207** and **IASO208**: For the indication r/r MM ($\geq 4L$), to submit IND applications to U.S. FDA and NMPA
- **IASO782**: For the indications ITP and wAIHA, Phase Ia patient enrollment to be completed

2025

2026

2027

- **CT103A**: For the indication r/r MM ($\geq 4L$), an application for full marketing approval submitted to the NMPA; BLA approved by ISAF, Macau; BLA accepted by HSA, Singapore; BLA accepted by DOH, Hong Kong; BLA accepted by SFDA, Saudi; BLA accepted by EDE, UAE; Received IND approval in Japan
- **IASO140**: For the indication r/r MM ($\geq 4L$), IIT patient enrollment to be completed in Q4 2025

- **CT103A**: For the indication r/r MM ($\geq 4L$), pivotal clinical trial to be completed, BLA to be submitted in 2028 based on the data collected; for the indication SLE/LN, patient enrollment to initiate in the U.S.
- **IASO206**: For the indication r/r MM ($\geq 4L$), to submit IND applications to U.S. FDA and NMPA
- **IASO782**: For the indication SLE, phase Ib patient enrollment to initiate in China

IASO in 2026

 **1 commercialized product**

CT103A, r/r MM ($\geq 4L$)

 **1 pipeline candidate in Phase III clinical trial**

CT103A, r/r MM (2L/3L), China

 **1 pipeline candidate in Phase II clinical trial**

IASO120, r/r B-NHL ($\geq 3L$), China

 **4 pipeline candidates in Phase I clinical trials**

CT103A, MG, China and USA
CT103A, MS, USA

IASO118, r/r MM ($\geq 4L$), China
IASO782, ITP and wAIHA, China

 **6 pipeline candidates in IIT clinical trials**

CT103A, NDMM (high-risk 1L), China
CT103A, MS/SLE/IIM etc., China
IASO140, r/r MM ($\geq 4L$), China

IASO206, r/r MM ($\geq 4L$), China
IASO207, r/r MM ($\geq 4L$), China
IASO208, r/r MM ($\geq 4L$), China

 **4 pipeline candidates in other phases of clinical trial**

CT103A, SLE/LN, China
IASO209, Undisclosed

IASO210, Undisclosed
IASO782, SLE, China

Overview of Investment Highlights



1

An Innovative Pioneer in Advanced ATMPs Built upon China's Global Excellence in Cell Therapy

2

Eque-cel: First-in-class and Potentially Best-in-class BCMA CAR-T with Promising Data in r/r MM Validated by NMPA Approval and Significant Indication Expansion Potential in Frontline MM and Autoimmune Diseases

3

Diversified, Comprehensive and Balanced Product Pipeline Developed with a Forward-Looking, De-risked Development Approach

4

Integrated End-to-end Development Capabilities Covering a Full Cycle of Innovative Drug R&D, and CMC and Clinical Development

5

A Proven Commercialization Platform and Expanding Global Footprint Demonstrating Strong Execution Capability and International Competitiveness

6

A Visionary Management Team With World-class Experts, Industry Veterans and Seasoned Entrepreneurs Backed by Reputable Investors



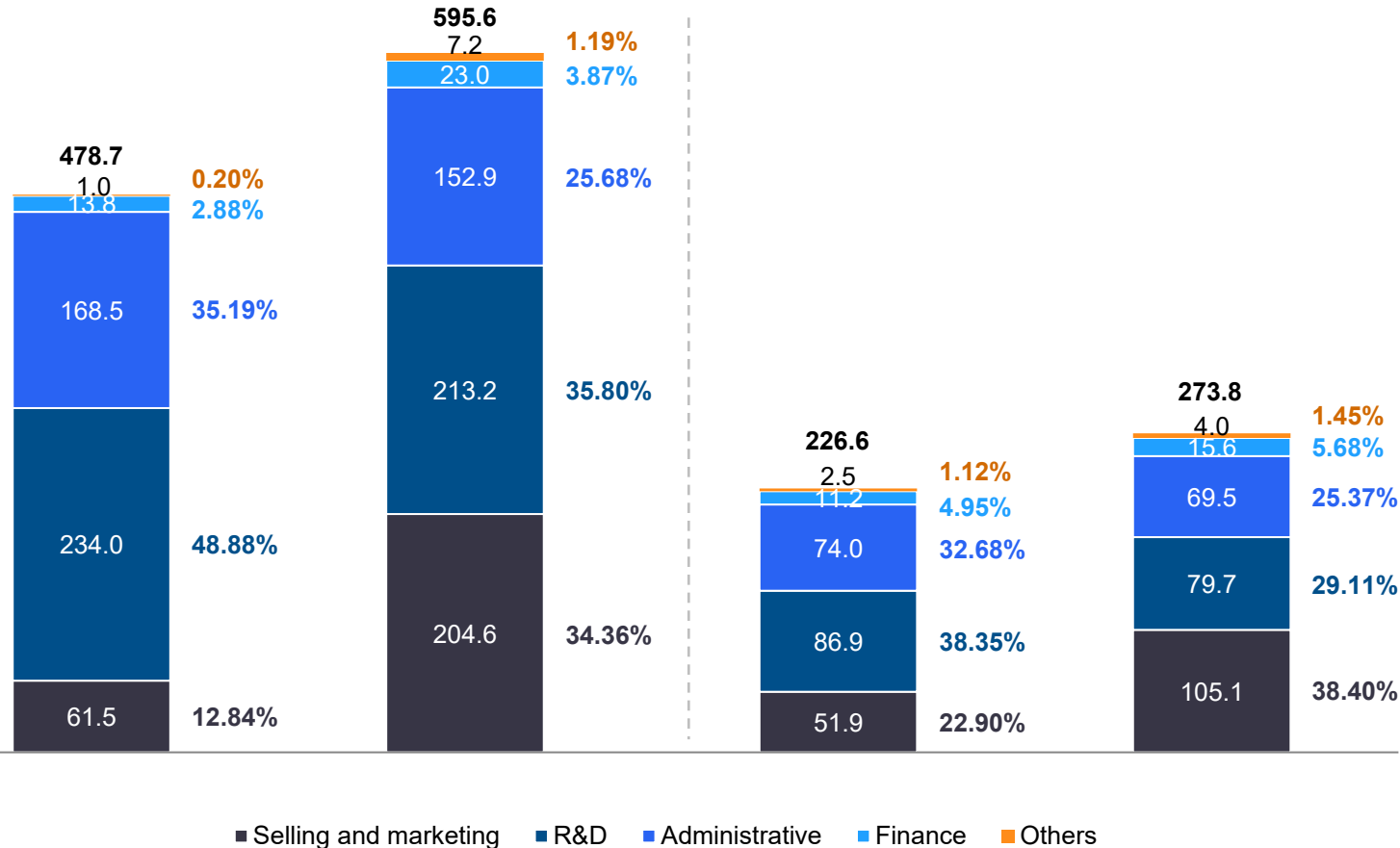
APPENDIX A

Financial Statements and Use of Proceeds

Expense Overview

2023 – 1H 2025 Expense Breakdown

(in million RMB)



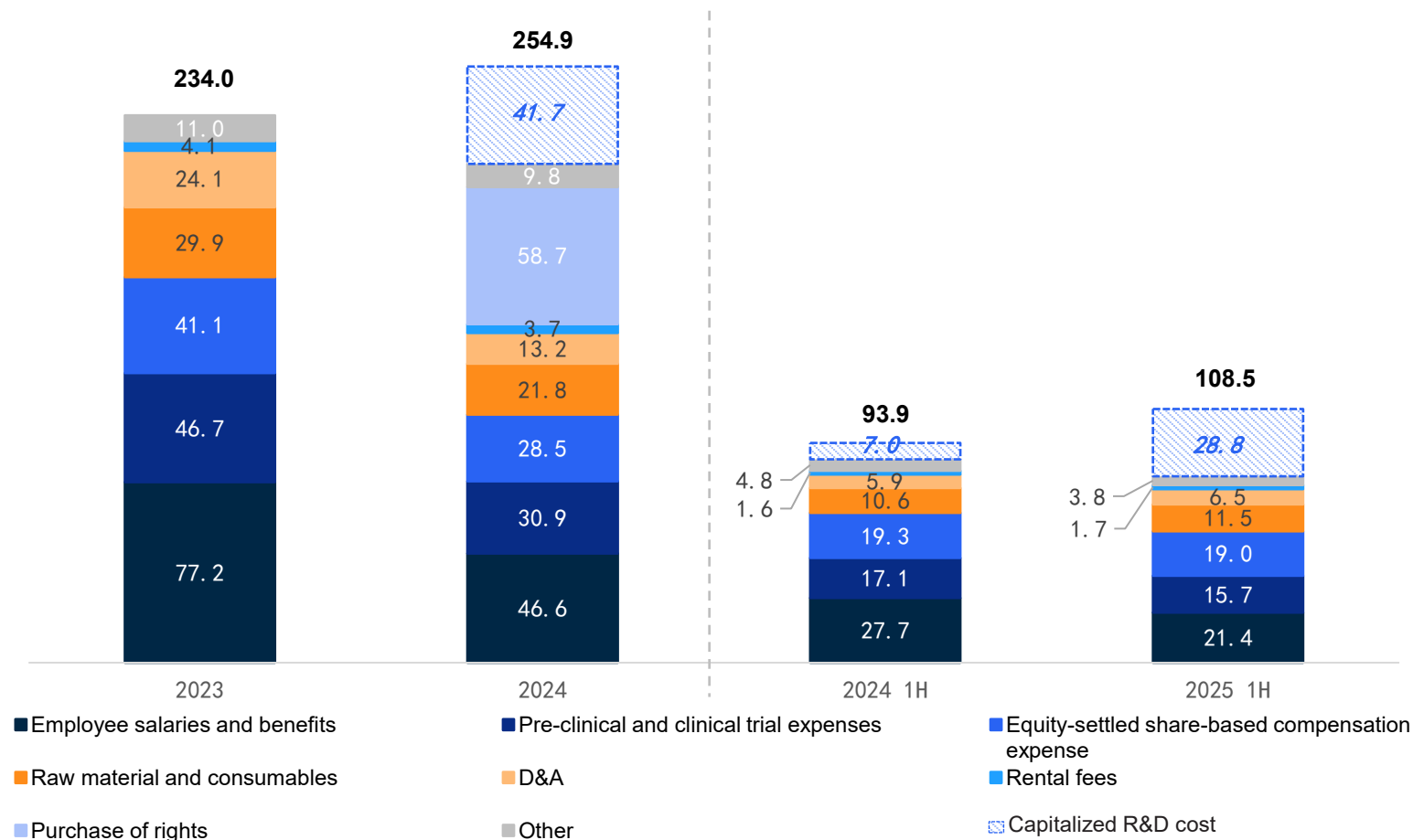
Expense Analysis

- During the Track Record Period, our operating expenses include selling and marketing expenses, R&D costs, administrative expenses.
- Going forward, our operating expenses are expected to evolve as we continue to progress and expand our pipeline and gradually bring our product pipeline to commercialization, and we expect an increase in production volume and continuing **improvements in cost efficiency** driven by the advantages of economies of scale.

R&D Costs

2023 – 1H 2025 R&D Cost Breakdown

(in million RMB)



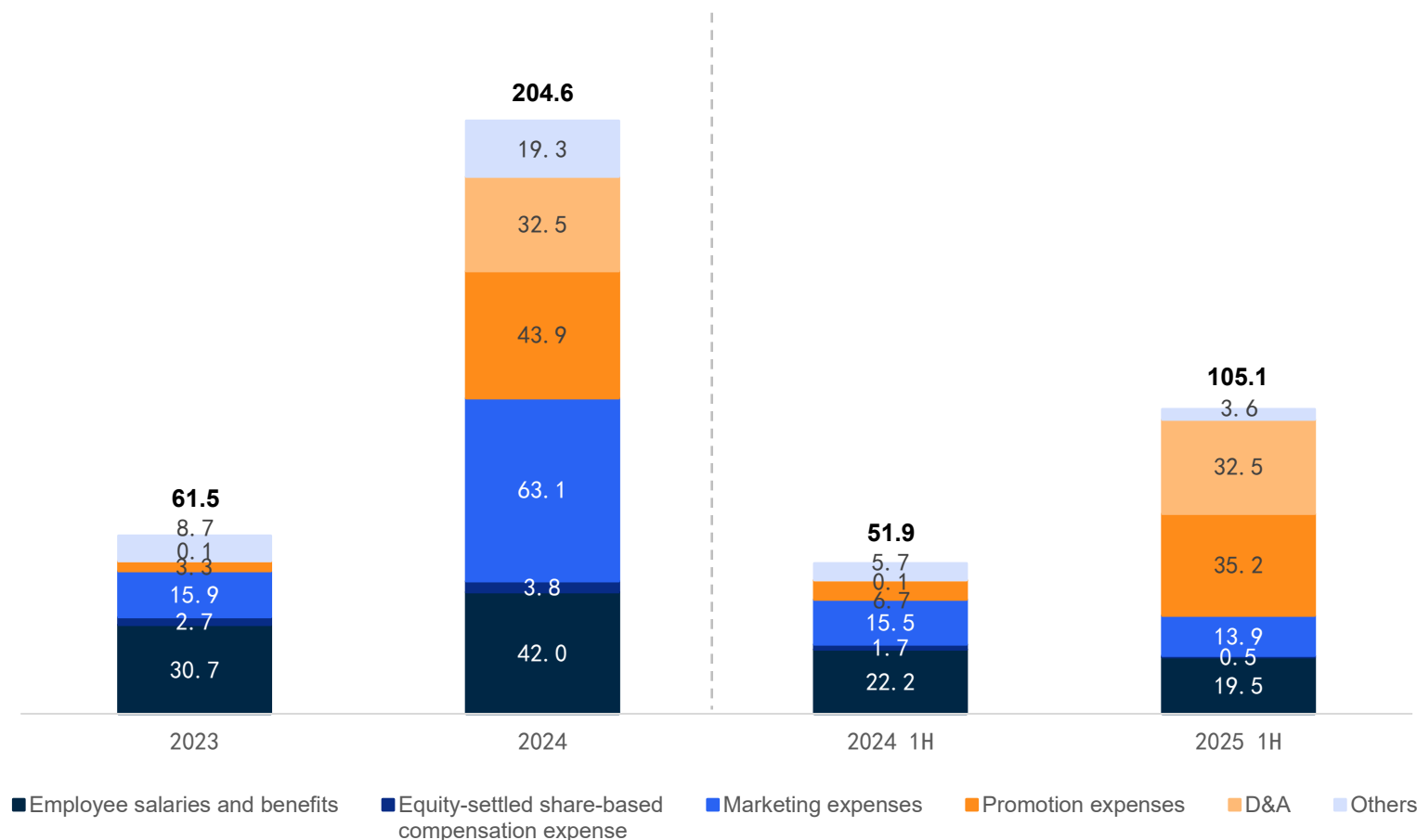
R&D Cost Analysis

- The R&D costs attributable to our Core Product account for **47.1%**, **67.8%**, **48.2%**, and **61.7%** of our total R&D costs.
- In addition, we commenced capitalization R&D costs in relation to certain pipelines of our Core Product since 2024, based on the development stage. The capitalized amounts were RMB41.7 million, RMB7.0 million and RMB28.8 million in 2024 and the six months ended June 30, 2024 and 2025.

Selling and Marketing Expenses

2023 – 1H 2025 Selling and Marketing Breakdown

(in million RMB)



S&M Expenses Analysis

- Our selling and marketing expenses primarily consisted of (i) employee salaries and benefits for our sales personnel; (ii) equity-settled share-based compensation expense; (iii) marketing expenses for academic activities, physician education and other marketing activities; (iv) promotion expenses, primarily consist of promotional activities for our Fucaso®; (v) D&A expenses for property, plant and equipment and intangible assets used for selling and marketing purpose.
- Particularly, we capitalized commercialization rights in relation to the asset purchase agreement we entered with Innovent in 2024, which was subsequently amortized as selling and marketing expenses.

Consolidated Statements of Profit or Loss

(RMB'000)	Year ended 31 December		Six months ended 30 June	
	2023	2024	2024 (unaudited)	2025 (unaudited)
REVENUE	44,786	207,478	93,394	90,957
Cost of sales	(19,949)	(66,478)	(26,946)	(29,237)
Gross profit	24,837	141,000	66,448	61,720
Other income and gains	29,885	56,778	21,403	15,710
Selling and marketing expenses	(61,485)	(204,621)	(51,877)	(105,138)
Administrative expenses	(168,482)	(152,931)	(74,034)	(69,459)
Research and development costs	(234,034)	(213,199)	(86,895)	(79,703)
Other expenses	(978)	(7,161)	(2,544)	(3,961)
Finance costs	(13,766)	(23,039)	(11,221)	(15,559)
Share of losses of an associate	—	(65)	—	—
LOSS BEFORE TAX	(424,023)	(403,238)	(138,720)	(196,390)
Income tax expenses	(439)	—	—	—
LOSS FOR THE YEAR/PERIOD AND ATTRIBUTABLE TO OWNERS OF THE COMPANY	(424,462)	(403,238)	(138,720)	(196,390)
LOSS PER SHARE ATTRIBUTABLE TO ORDINARY EQUITY HOLDERS OF THE COMPANY (RMB)	-1.14	-0.97	-0.37	-0.43

Consolidated Statements of Financial Position

(RMB'000)	As at 31 December		As at 30 June
	2023	2024	2025 (unaudited)
NON-CURRENT ASSETS			
Property, plant and equipment	99,764	67,464	84,075
Right-of-use assets	218,350	207,871	197,479
Other intangible assets	6,417	854,685	849,833
Investment in an associate	–	1,935	–
Time deposits	183,824	177,288	179,613
Other non-current assets	3,098	7,009	3,190
Total non-current assets	511,453	1,316,252	1,314,190
CURRENT ASSETS			
Inventories	25,776	33,369	43,340
Trade receivables	1,046	2,972	846
Prepayments, other receivables and other assets	44,812	97,848	116,303
Financial assets at fair value through profit or loss	50,225	–	–
Restricted cash	1,949	2,045	2,045
Time deposits	121,755	190,652	337,664
Cash and cash equivalents	179,163	57,626	53,039
Total current assets	424,726	384,512	553,237
TOTAL ASSETS	936,179	1,700,764	1,867,427

Consolidated Statements of Financial Position (cont'd)

(RMB'000)	As at 31 December		As at 30 June
	2023	2024	2025 (unaudited)
CURRENT LIABILITIES			
Trade payables	24,826	26,686	32,766
Other payables and accruals	31,577	40,053	37,463
Contract liabilities	21,549	28,584	29,072
Interest-bearing bank borrowings	54,532	45,170	67,214
Lease liabilities	4,761	17,990	17,051
Total current liabilities	137,245	158,483	183,566
NET CURRENT ASSETS	287,481	226,029	369,671
TOTAL ASSETS LESS CURRENT LIABILITIES	798,934	1,542,281	1,683,861
NON-CURRENT LIABILITIES			
Interest-bearing bank borrowings	206,050	377,880	685,195
Lease liabilities	238,833	240,514	237,827
Deferred income	3,659	2,768	2,266
Total non-current liabilities	448,542	621,162	925,288
EQUITY	350,392	921,119	758,573
Share capital	373,372	455,332	455,332
(Deficits)/reserves	(22,980)	465,787	303,241
Total equity	350,392	921,119	758,573

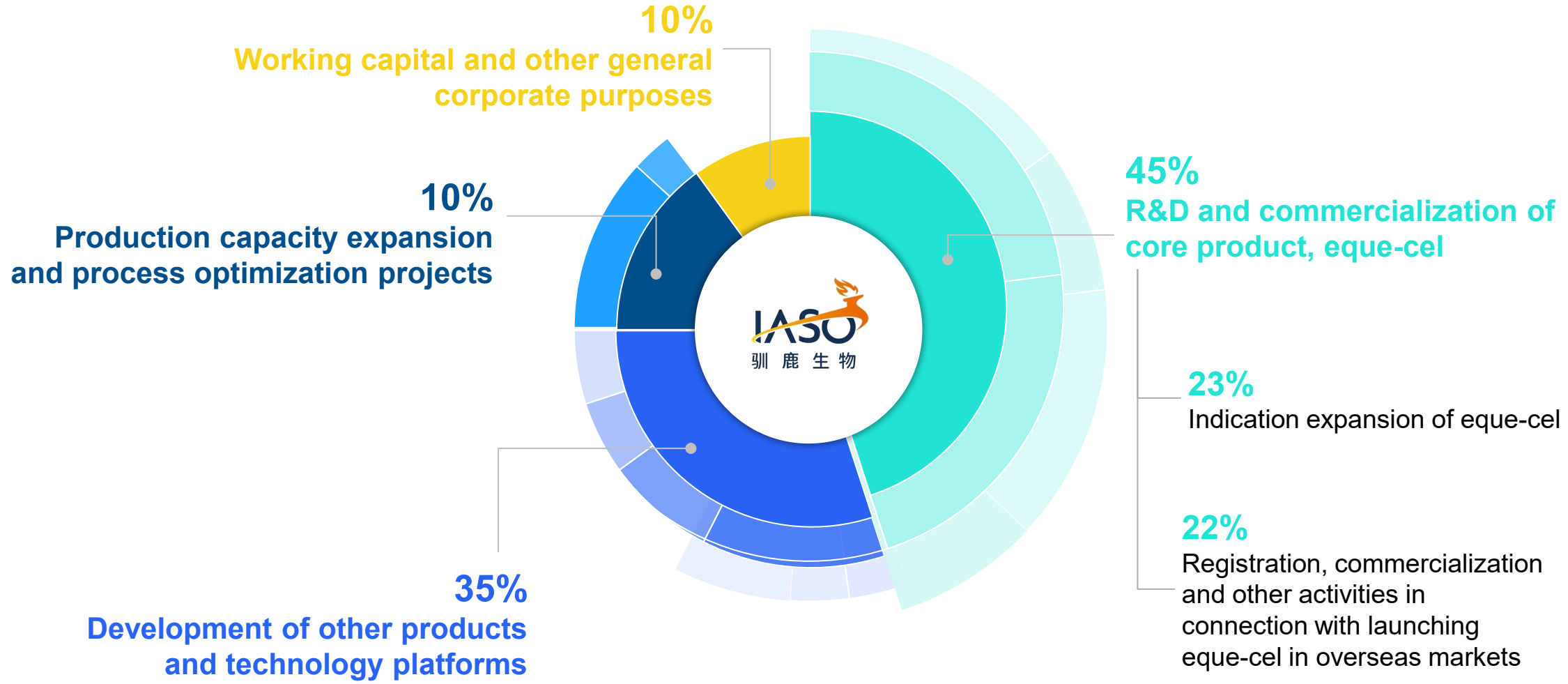
Consolidated Statements of Cash Flows

(RMB'000)	Year ended 31 December		Six months ended 30 June	
	2023	2024	2024 (unaudited)	2025 (unaudited)
CASH FLOWS FROM OPERATING ACTIVITIES				
Loss before tax	(424,023)	(403,238)	(138,720)	(196,390)
Adjustments for:				
Finance costs	13,766	23,039	11,221	15,559
Bank interest income	(12,996)	(16,494)	(9,099)	(9,112)
Other investment income from financial assets at fair value through profit or loss	(2,365)	(476)	(357)	–
Gain from disposal of an associate	–	–	–	(65)
Depreciation of property, plant and equipment	39,717	38,569	22,554	33,548
Depreciation of right-of-use assets	14,318	20,353	10,003	10,392
Amortisation of intangible assets	2,758	35,325	1,433	33,897
Loss on disposal of property, plant and equipment	58	10	8	11
Share of losses of an associate	–	65	–	–
Fair value gain on financial assets at fair value through profit or loss	(225)	–	–	–
Equity-settled share-based compensation expense	106,235	73,853	44,362	33,832
Increase in inventories	(16,279)	(7,593)	(10,211)	(9,971)
Decrease/(increase) in prepayments, other receivables and other assets	21,850	(53,036)	(19,053)	(18,455)
(Increase)/decrease in trade receivables	(1,046)	(1,926)	300	2,126
(Decrease)/increase in trade payables	(9,982)	1,860	10,659	6,080
Increase in contract liabilities	21,549	7,035	4,778	488
Decrease in deferred income	(838)	(891)	(427)	(502)
Increase/(decrease) in other payables and accruals	(8,102)	8,991	(1,901)	725
Cash used in operations	(255,605)	(274,554)	(74,450)	(97,837)
Interest received	2,871	2,507	1,164	277
Income tax paid	(439)	–	–	–
Net cash flows used in operating activities	(253,173)	(272,047)	(73,286)	(97,560)

Consolidated Statements of Cash Flows (cont'd)

(RMB'000)	Year ended 31 December		Six months ended 30 June	
	2023	2024	2024 (unaudited)	2025 (unaudited)
CASH FLOWS FROM INVESTING ACTIVITIES				
Purchases of items of property, plant and equipment	(21,913)	(10,792)	(5,531)	(48,095)
Proceeds from disposal of items of property, plant and equipment and other intangible assets	278	108	72	839
Purchases of other intangible assets	(987)	(883,594)	(7,004)	(29,045)
Purchases of financial assets at fair value through profit or loss	(955,000)	(340,000)	(240,000)	–
Increase in bank deposits with original maturity of more than three months when acquired	(550,368)	(207,846)	(96,252)	(208,647)
Proceeds of financial assets at fair value through profit or loss	907,365	390,701	290,582	–
Withdrawal of bank deposits with original maturity of more than three months when acquired	254,915	159,471	17,593	68,145
Proceeds from disposal of an associate	–	–	–	2,000
Purchase of a shareholding in an associate	–	(2,000)	–	–
Net cash flows used in investing activities	(365,710)	(893,952)	(40,540)	(214,803)
CASH FLOWS FROM FINANCING ACTIVITIES				
Net proceeds of capital injection from shareholders	418,779	900,000	–	–
New bank loans	232,000	337,300	252,765	569,740
Repayment of bank loans	(128,981)	(182,350)	(146,808)	(240,855)
Principal portion of lease payments	(2,202)	(7,547)	(2,933)	(9,906)
Increase in restricted cash	(21)	(97)	–	–
Interest paid	(15,036)	(2,938)	(4,537)	(8,805)
Payments on listing expenditures	–	–	–	(2,295)
Net cash flows from financing activities	504,539	1,044,368	98,487	307,879
Net increase in cash and cash equivalents	(114,344)	(121,631)	(15,339)	(4,484)
Cash and cash equivalents at beginning of year/period	293,371	179,163	179,163	57,626
Effect of foreign exchange rate changes, net	136	94	270	(103)
CASH AND CASH EQUIVALENTS AT END OF YEAR/PERIOD	179,163	57,626	164,094	53,039

Use of Proceeds

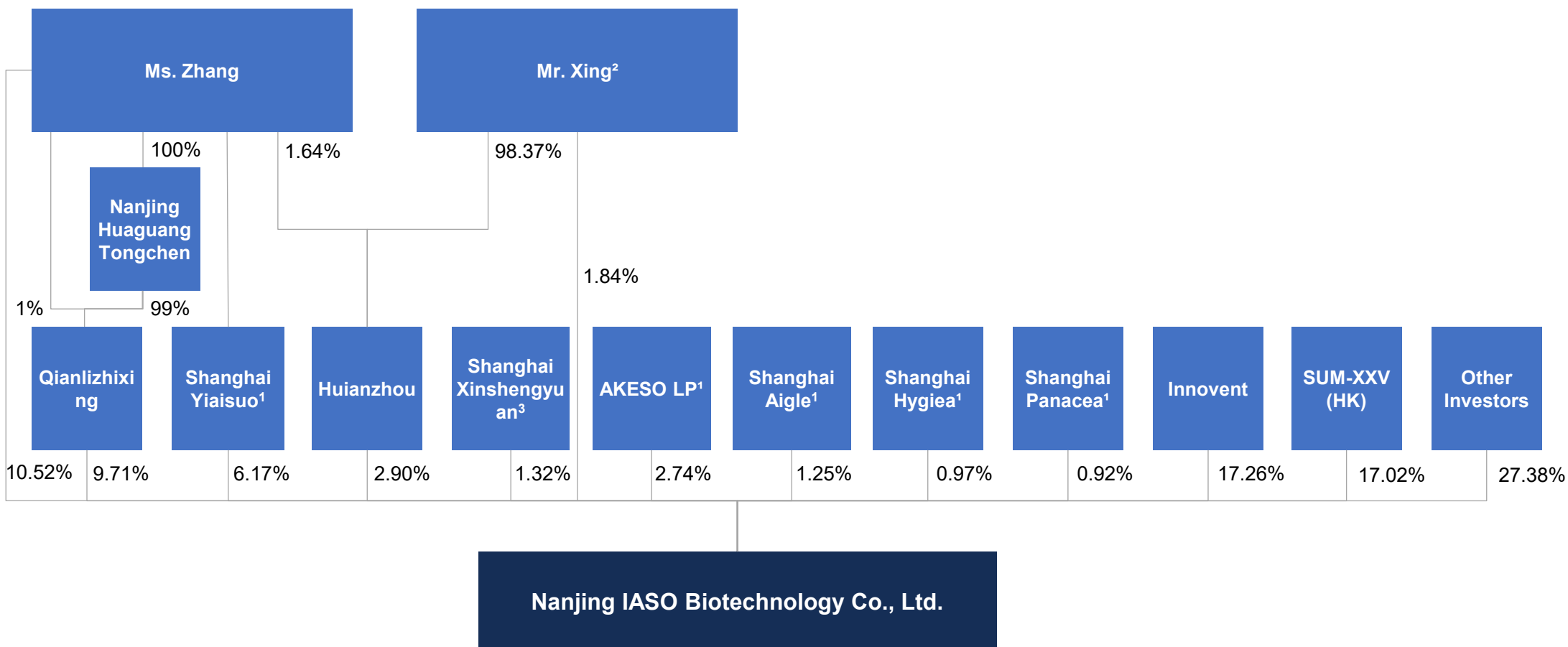




APPENDIX B

Shareholding Structure

Corporate Structure



Notes:

1. Each of Shanghai Yiaisuo, AKESO LP, Shanghai Aigle, Shanghai Hygiea and Shanghai Panacea is an employee shareholding platform of the Company
2. Mr. Xing is an Independent Third Party to the Company
3. The general partner of Shanghai Xinshengyuan is Mr. Zou, the non-executive Director of our Company

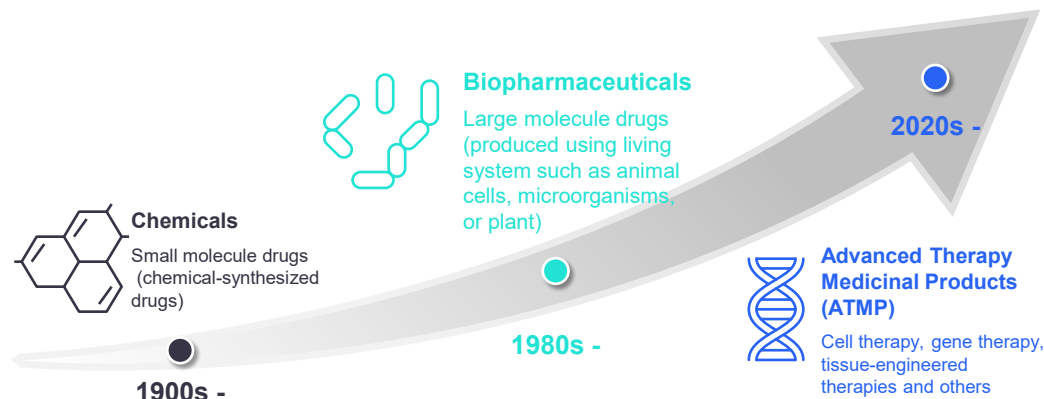


APPENDIX C

Market Data

ATMP & CGT Definitions and Market Landscape

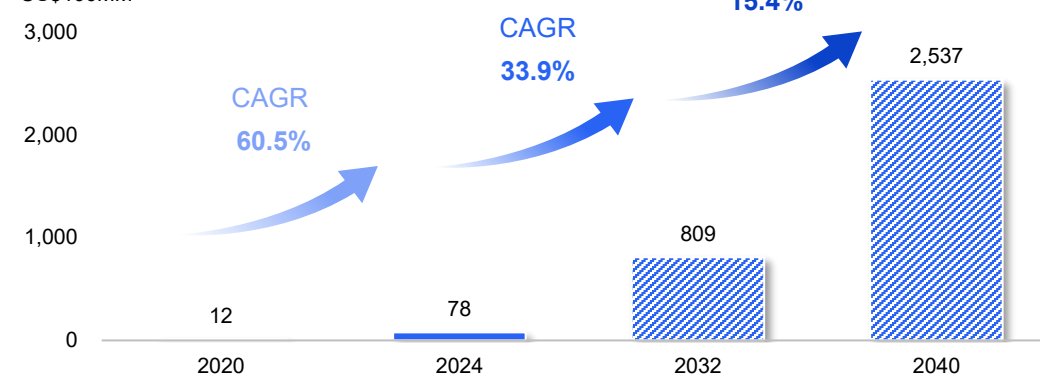
ATMP Has Become a Core Strategic Focus in Global Biotech, with CGT Being the Most Important Sector



Unlike conventional drugs such as small molecules and other biologics that modulate disease pathways or alleviating symptoms, ATMPs aim to address the root causes of disease by regenerating cells or tissues, correcting genetic defects, and achieving functional cures

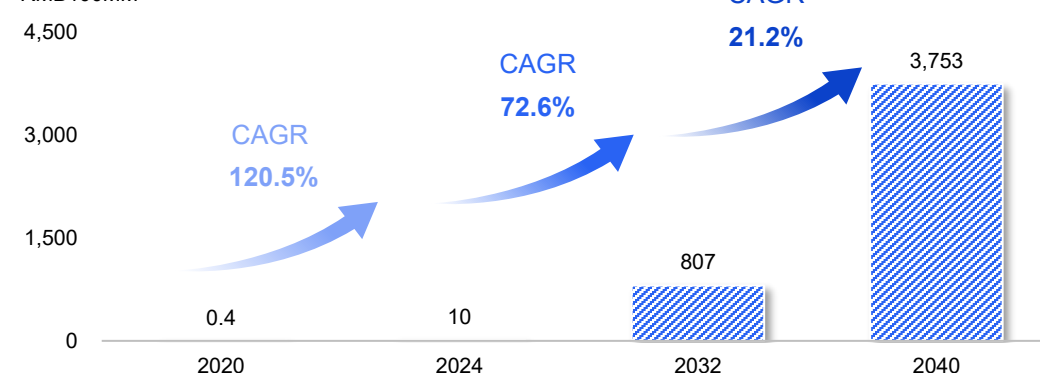
Global Market Size of CGT

US\$100MM



Market Size of CGT in China

RMB100MM



- ATMPs — including **cell therapies**, **gene therapies**, and **tissue-engineered products** represent a new therapeutic frontier
- China has established notable advantages in the development of ATMP products, positioning itself as a global leader, with more than **1,300** registered clinical trials as of 2024 spanning CAR-T, stem cell, and other somatic cell therapies



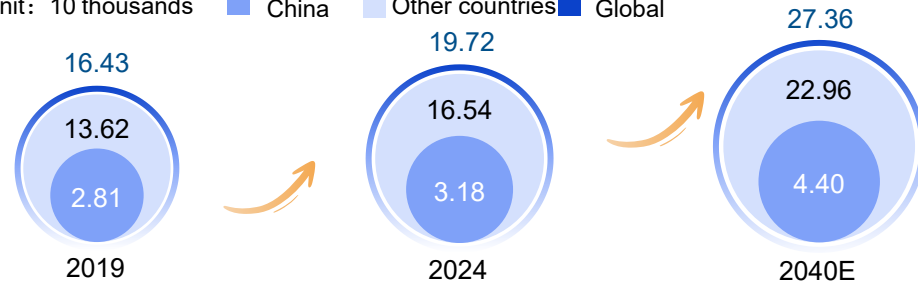
- CGT (*cell and gene therapy*), which represents the most important sector within the ATMP landscape, includes:
 - cell-based therapies**, such as **CAR-T cell therapies**
 - gene therapies that target the underlying genetic causes of diseases or modifying genes through gene addition, gene silencing or gene editing by using viral or non-viral vectors
- The U.S. FDA has approved **seven** CAR-T cell therapies and the NMPA has approved **eight**

Source: Frost & Sullivan

MM industry overview

MM is a cancer of plasma cells in the bone marrow. Plasma cells are antibody-producing white blood cells that play a critical role in the immune system. MM develops when healthy plasma cells become malignant and proliferate uncontrollably. The disease often leads to significant skeletal damage, including osteolytic lesions, osteopenia, and pathological fractures.

Unit: 10 thousands ■ China ■ Other countries ■ Global

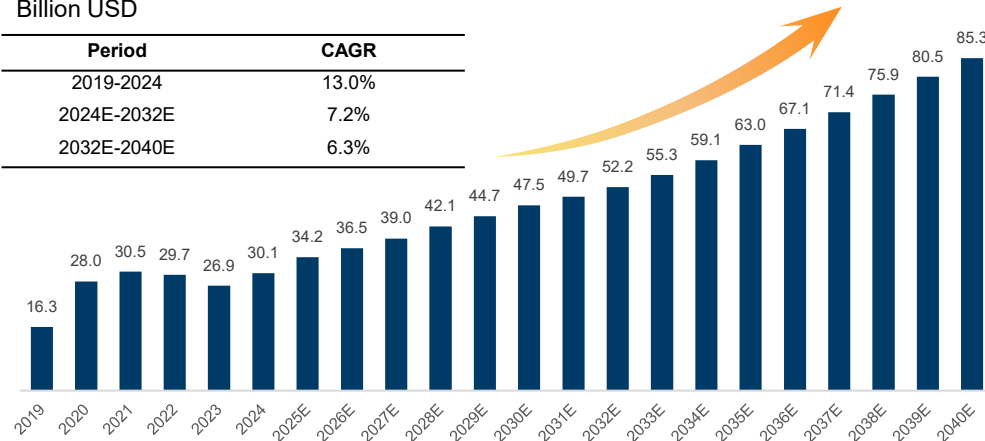


- Globally, the incidence of MM increased from 164.3 thousand cases in 2019 to 197.2 thousand in 2024 and is projected to reach **273.6 thousand** by 2040. In China, incidence rose from 28.1 thousand in 2019 to 31.8 thousand in 2024 and is expected to reach **44.0 thousand** by 2040.
- CAR-T cell therapies targeting plasma cell-associated antigens, including **BCMA**, have emerged as a critical strategy for the treatment of MM. **GPRC5D** has recently emerged as a promising alternative target. It is expressed independently of BCMA and is highly enriched on malignant plasma cells, suggesting a favorable therapeutic index.

Global MM Drugs Market Size, 2019 - 2040E

Billion USD

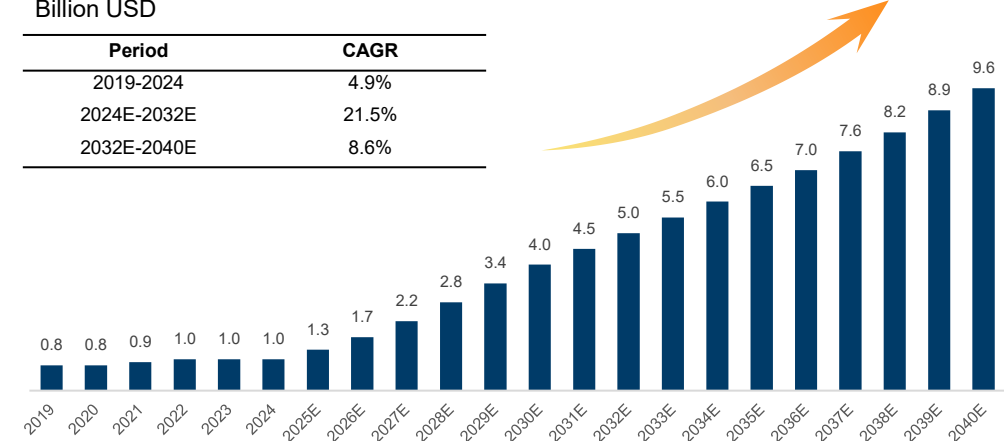
Period	CAGR
2019-2024	13.0%
2024E-2032E	7.2%
2032E-2040E	6.3%



China MM Drugs Market Size, 2019 - 2040E

Billion USD

Period	CAGR
2019-2024	4.9%
2024E-2032E	21.5%
2032E-2040E	8.6%



--While CAR-T cell therapy has transformed the treatment landscape of multiple MM, critical unmet needs remain.
 --Despite its approval in numerous countries and regions globally, the availability and accessibility of CAR-T therapy are still limited beyond key markets such as Singapore, Thailand, and many European nations.

MM Competitive Landscape

Generic Name	Brand Name	Target	Company	First Approval Date	Fully Human	Approved Region	Market Share in China
Equyecabtagene autoleucel (伊基奥仑赛注射液)	Fucaso® 福可苏®	BCMA	The Company	2023-06-30	Yes	Chinese mainland, Macau	67.7%
Zevorcabtagene autoleucel (泽沃基奥仑赛注射液)	赛恺泽®	BCMA	CARsgen Therapeutics	2024-02-23	Yes	Chinese mainland	32.3%
Idecabtagene vicleucel	Abecma®	BCMA	BMS	2021-03-03	No	EU, Japan, U.S., UK, Switzerland, Israel	/
Ciltacabtagene autoleucel (西达基奥仑赛注射液)	Carvykti® 卡卫获®	BCMA	Johnson/LEGEND BIOTECH	2022-02-28 (U.S.) 2024-08-20 (Chinese mainland)	No	U.S., EU, UK, Chinese mainland, South Korea, Canada, Japan, Brazil, Australia	/

Notes: Carvykti® was approved but not yet laur

Brand	Fucaso®		Abecma®	Carvykti®	赛恺泽®
	Patients without prior CAR-T, N=97	All patients N=109	N=128	N=97	N=102
Approval	China Jun. 2023		USA Mar. 2021	UAS Feb. 2022	China Feb.2024
Overall Response	98.9%*	96.3%**	73%	97.9%	92.2%
≥Complete Remission	88.4%*	83.2%**	33%	82.5%	71.6%
mPFS (months)	35.9	30.5	8.8	34.9	/
≥G3 CRS	1.0%	0.9%	5.5%	5.1%	6.9%
≥G3 ICANS	0%		/	2.1%	0%
Parkinson's syndrome	0%		/	5%	0%
Second Primary Malignancies	0%		7%	6%	0%
≥G3 CAR-T associated neurotoxicity (including ICANS)	0%		4%	11%	0%

- Among the CAR-T cell therapies approved globally, according to the latest data as of December 31, 2024, **Fucaso® showed the highest overall response rate at 98.9% and the CR/sCR rate at 88.4%** among CAR-T therapy treatment naïve patients.
- In terms of safety, Fucaso® has the **lowest incidence of severe cytokine release syndrome and no reported cases of severe neurotoxicity**, highlighting its strong efficacy coupled with an excellent safety profile compared to its peers.

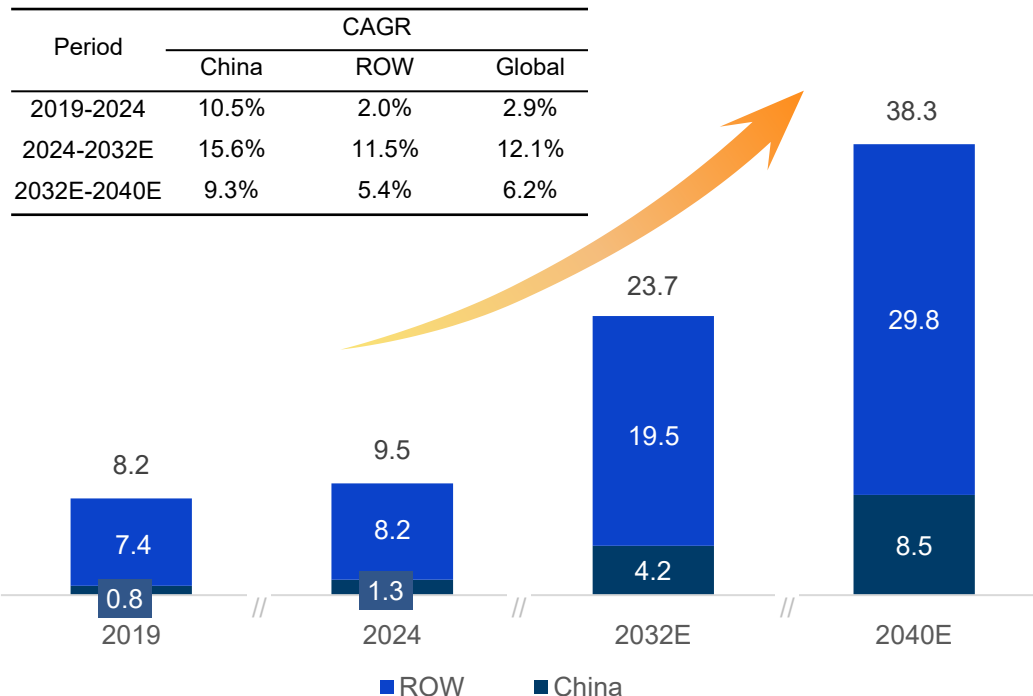
*Data collected from 95 efficacy evaluable patients, as of data cut-off date December 31, 2024. **Data collected from 107 efficacy evaluable patients, as of data cut-off date December 31, 2024.

NHL Market Overview

NHL accounts for approximately 90% of all lymphomas and encompasses a wide range of subtypes. Aggressive B-cell lymphomas are a subset of NHLs that grow rapidly and require prompt, intensive treatment. The most common subtype is **diffuse large B-cell lymphoma (“DLBCL”)**

Historical and Forecasted Global Market Size of DLBCL drugs, 2019-2040E

Billion USD



Competitive Landscape of CAR-T Cell Therapies for DLBCL

- ✓ There were six CAR-T cell therapies approved for DLBCL treatment on a global scale, all the products solely targeted CD19.
- ✓ IASO120 stands out as the most clinically advanced bispecific CAR-T cell therapy for DLBCL in China.

Drug Name	Target	Clinical Stage	Indications	Company	First Posted Date	Region
Rondecabtagene autoleucel	CD19/CD20	Phase III	DLBCL	ImmPACT Bio	2025-09-23	/
Zamtocabtagene autoleucel	CD19/CD20	Phase II	DLBCL	Miltenyi Biomedicine	2021-03-09	Global
IASO120	CD19/CD22	Phase I/II	DLBCL	The Company	2021-09-15	China
AVC-203	CD19/CD20	Phase I/II	DLBCL	AvenCell	2025-10-27	EU
EB BH2024	CD19/CD22	Phase I/II	DLBCL	Essen Biotech	2024-01-19	China
HY004	CD19/CD22	Phase I/II	DLBCL	Juventas Cell	2023-08-30	China
EXP039	CD19/CD20	Phase I/II	DLBCL	AbelZeta	2022-06-16	Global

Significant unmet medical need remains in the treatment of DLBCL. Although CAR-T cell therapy has been recognized as a breakthrough treatment and has already been incorporated into second- and third-line treatment paradigms, its long-term efficacy remains limited. This highlights the urgent need for safer, more tolerable treatments.



APPENDIX D

Clinical Data

Eque-cel - First-in-class and Potentially Best-in-class BCMA CAR-T Therapy



Clinical Data Summary

Phase Ib/II Pivotal Clinical Trial of Eque-cel in Patients with r/r MM (FUMANBA-1)

Efficacy Profile

- Among 107 evaluable patients for efficacy assessment, a 96.3% ORR was observed, with 83.2% ≥CR. Median DOR and median PFS were 31.3 months and 30.5 months. For patients without prior CAR-T cell therapy (N = 95), ORR was 98.9%, while the median PFS reached 35.9 months
- The overall MRD negativity rate was 95.3%. Among patients who achieved a CR or better (≥CR, N=89), the MRD negativity rate was 100%. In patients who had not received prior CAR-T cell therapy, the MRD negativity rate was 98.9%. The median duration of MRD negativity was 36.5 months, in both patients who achieved ≥CR and in those without prior CAR-T cell therapy

Safety Profile

- CRS occurred in 93.6% of the 109 treated patients, but the majority (92.7%) were Grade 1 to 2, with only 1 patient (0.9%) experiencing Grade 3 CRS.
- The median onset of CRS was 6 days, and the median duration was 5 days. ICANS was rare, affecting only 2 patients (1.8%), both Grade 1 to 2

PFS Kaplan-Meier Curve

Median (95%CI)
Total 30.46(24.11, 42.15)
sCR/CR 39.82(30.26, NE)
Without prior CAR-T 35.91(25.95, 47.67)

No. at Risk	Total	sCR/CR	Without prior CAR-T
0	109	89	97
3	102	88	92
6	94	86	88
9	88	84	84
12	82	79	78
15	72	71	68
18	70	69	66
21	60	60	56
24	57	57	53
27	51	51	49
30	48	48	46
33	40	40	38
36	40	40	38
39	31	31	29
42	24	24	22
45	13	13	11
48	6	6	4
51	4	4	3
54	2	2	2

MRD Kaplan-Meier Curve

Median MRD (95% CI) 36.53(25.56, NE)
12-month MRD rate (95% CI) 81.0 (71.2, 87.7)
24-month MRD rate (95% CI) 62.2 (50.6, 71.9)
36-month MRD rate (95% CI) 50.2 (37.9, 61.3)

No. at Risk	Total
0	109
3	102
6	94
9	88
12	82
15	72
18	70
21	60
24	57
27	51
30	48
33	40
36	31
39	24
42	13
45	6
48	4
51	2
54	0

MRD Kaplan-Meier Curve

N=109	CRS	ICANS
Any grade, n(%)	102 (93.6)	2 (1.8)
Grade 1	74 (67.9)	1 (0.9)
Grade 2	27 (67.9)	1 (0.9)
Grade ≥3	27 (24.8)	0
Time to first onset, days, median (range)	6 (1, 13)	6.5 (3, 10)
Duration, days, median (range)	5 (2, 30)	1.5 (1, 2)

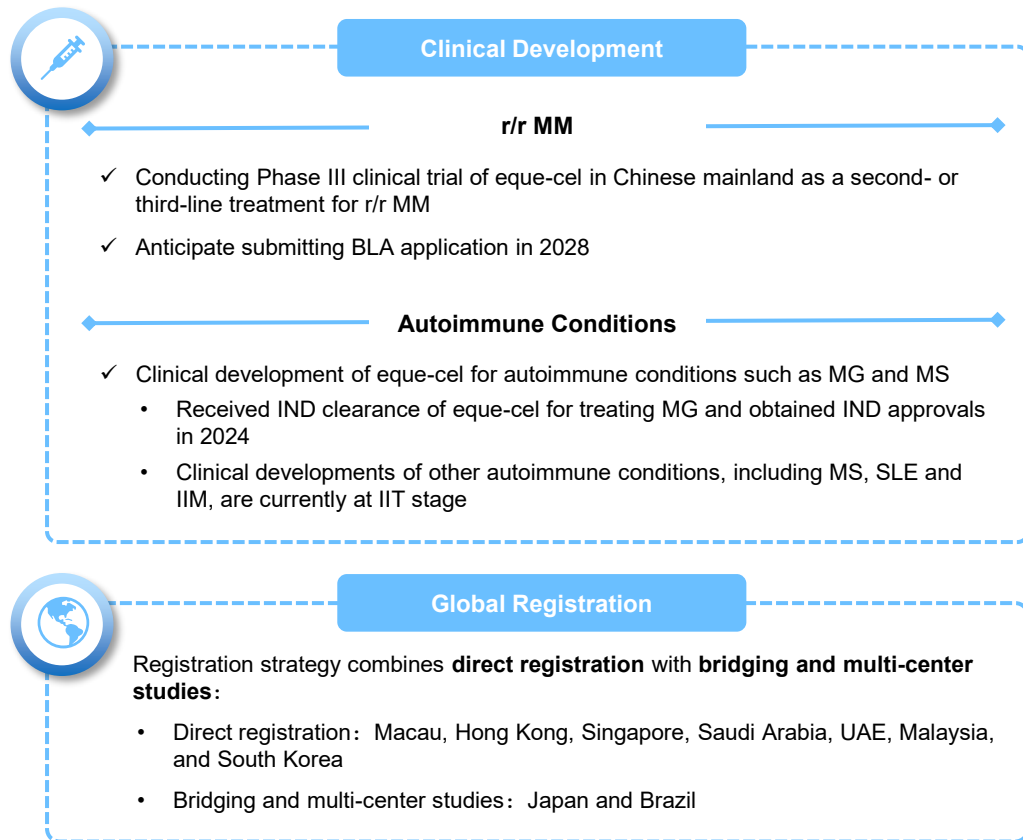


APPENDIX E

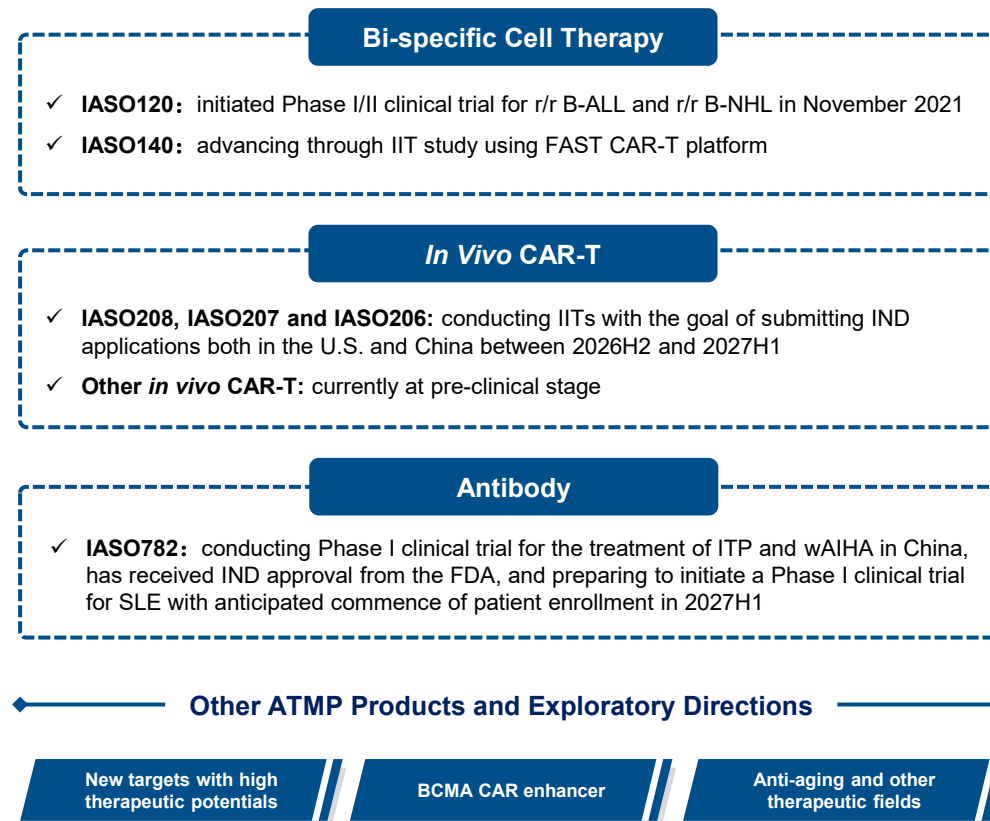
Corporate Strategy and Potential Milestones

Advance Clinical Development and Registration of Core Product and Build Next-generation Innovative Pipeline

Advance the Clinical Development and Registration of Eque-cel Globally to Further Unlock the Clinical and Commercial Value



Build Next-generation Pipeline under a Forward Looking Approach to Strengthen Our Competitive Position



Deepen Presence in the Domestic China Market, Strengthen Global Strategy and Explore Strategic Investment and Acquisition Opportunities

Expand Commercial Footprints in the Domestic China Market

Commercial Strategy

Enhance brand recognition

Expand marketing team

Strengthen geographic coverage

Strengthen full industry chain integrations

Deeper collaborations with hospitals

Facilitate communication among KOLs

Innovative Payment System

- ✓ Seek additional insurance coverage of our products and enhance marketing efforts

Continuous efforts to expand the coverage domestically by Huimin insurance, commercial insurance and installment payment programs

Implement a “Products out, Patients in” Global Strategy

Global Strategy

To capture the significant global potential of CAR-T cell therapies, we are executing an international strategy focused on expanding the global presence, laying the foundation for global market expansion

Promoting early international access through NPPs and patient-referral programs

Global Indication Expansion Strategy

Co-development, out-licensing, and in-licensing opportunities

Transforming current technology into a replicable international service platform

Explore Strategic Investment and Acquisition Opportunities on a Global Scale

Development Capability

Companies with ATMP development capabilities and cutting edge technologies

Business Synergy

Businesses with synergies in products, pipeline and industry chain to facilitate expansion of our therapeutic areas

Market Promotion

Companies with comprehensive market coverage to help us expand commercial footprint and facilitate global expansion

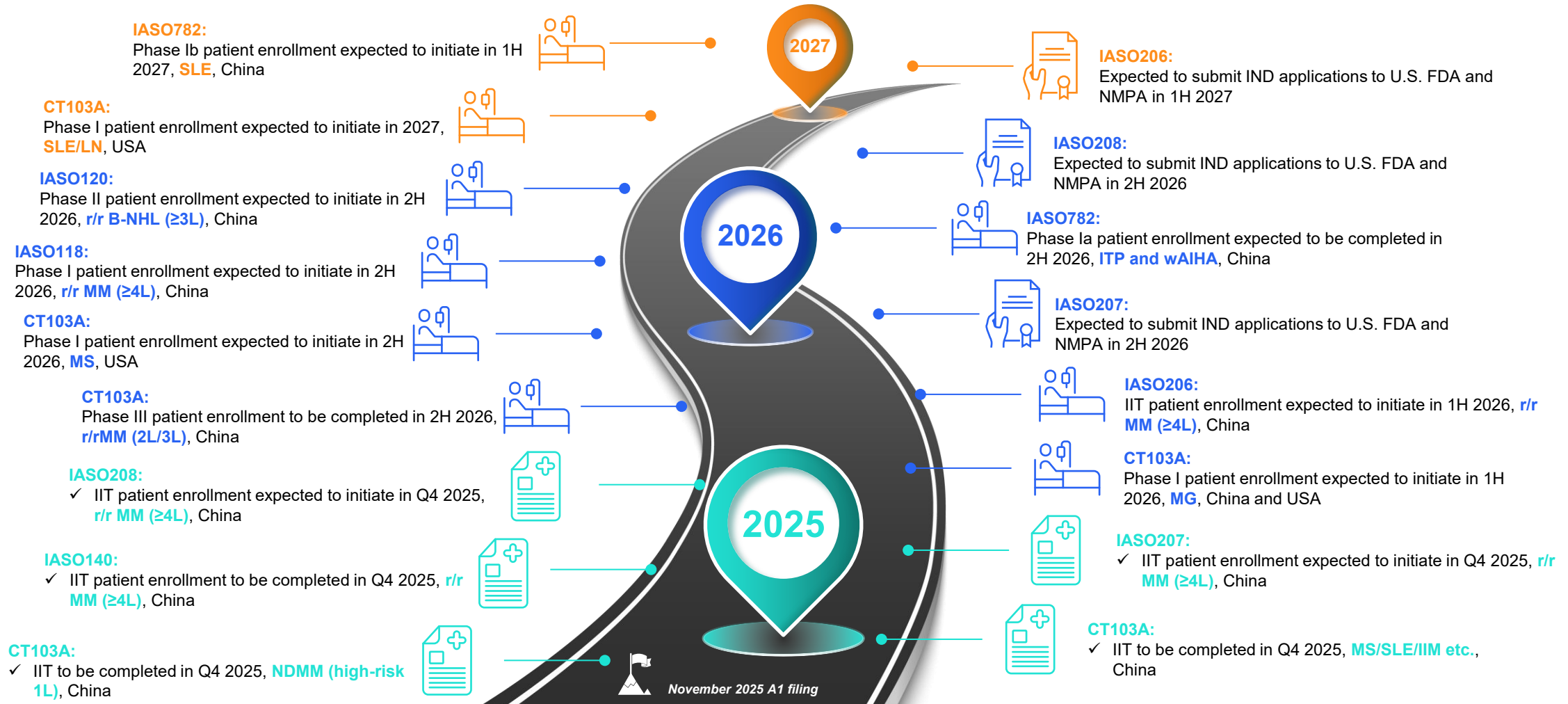
Sino-US Collaboration on Joint R&D Efforts

Efficient Transition from Early-Stage Development to Global Clinical Trials

- We are executing a disciplined global expansion strategy, leveraging complementary strengths of our R&D centers in China and the United States to advance projects rapidly, and pursuing clinical and regulatory pathways in more than 10 regions
- In 2021, our U.S. R&D laboratory commenced operation; in 2025, our global R&D center in Shanghai commenced operation



Expected Next Milestone of Pipeline





Thank You